

“Controlled Substance Prescribing and the Tennessee Chronic Pain Guidelines”

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Summit Medical Group

Knoxville, TN

“Controlled Substance Prescribing and the Tennessee Chronic Pain Guidelines”

Board Certified, Family Medicine 1989, meeting MOC Requirements

Fellow, American Academy of Family Physicians

Member, Tennessee Board of Medical Examiners 2013 – Present

Immediate Past President – Current

President – 2019-2020

Vice President – 2017-2018

Chair Development Committee 2017-2018

“Controlled Substance Prescribing and the Tennessee Chronic Pain Guidelines”

Most Slides Provided by and a Special Thanks to:

Michael Baron, MD, MPH, DFASAM
Medical Director

Tennessee Medical
Foundation – Physician’s
Health Program



“Controlled Substance Prescribing and the Tennessee Chronic Pain Guidelines”

OBJECTIVES

Review the Controlled Substance Monitoring Database (CSMD).

List new laws pertaining to CSMD including electronic prescribing of controlled substances.

Review the State of Tennessee guidelines on opioid prescribing.

Discuss the concept of MEDD.

Discuss Tennessee Chronic Pain Guidelines updates, outcomes of previous efforts in the state and be able to visualize current prescription related trends.

Demonstrate an understanding of prescription opioid misuse and abuse trends.

Identify strategies to incorporate evidence-based best practice.

Describe the two items needed on the Notice and Formulary

“Controlled Substance Prescribing and the Tennessee Chronic Pain Guidelines”

OBJECTIVES

Identify the rules that guide a supervising physician and know where to find them.

Discuss methods to screen patients for **substance** use disorders (SUD).

Describe the concept of behind adverse childhood experiences (**ACE**).

Define **addiction**.

Discuss the **etiologies** of addiction. Identify the concepts and potencies of **non-opioid analgesics**.

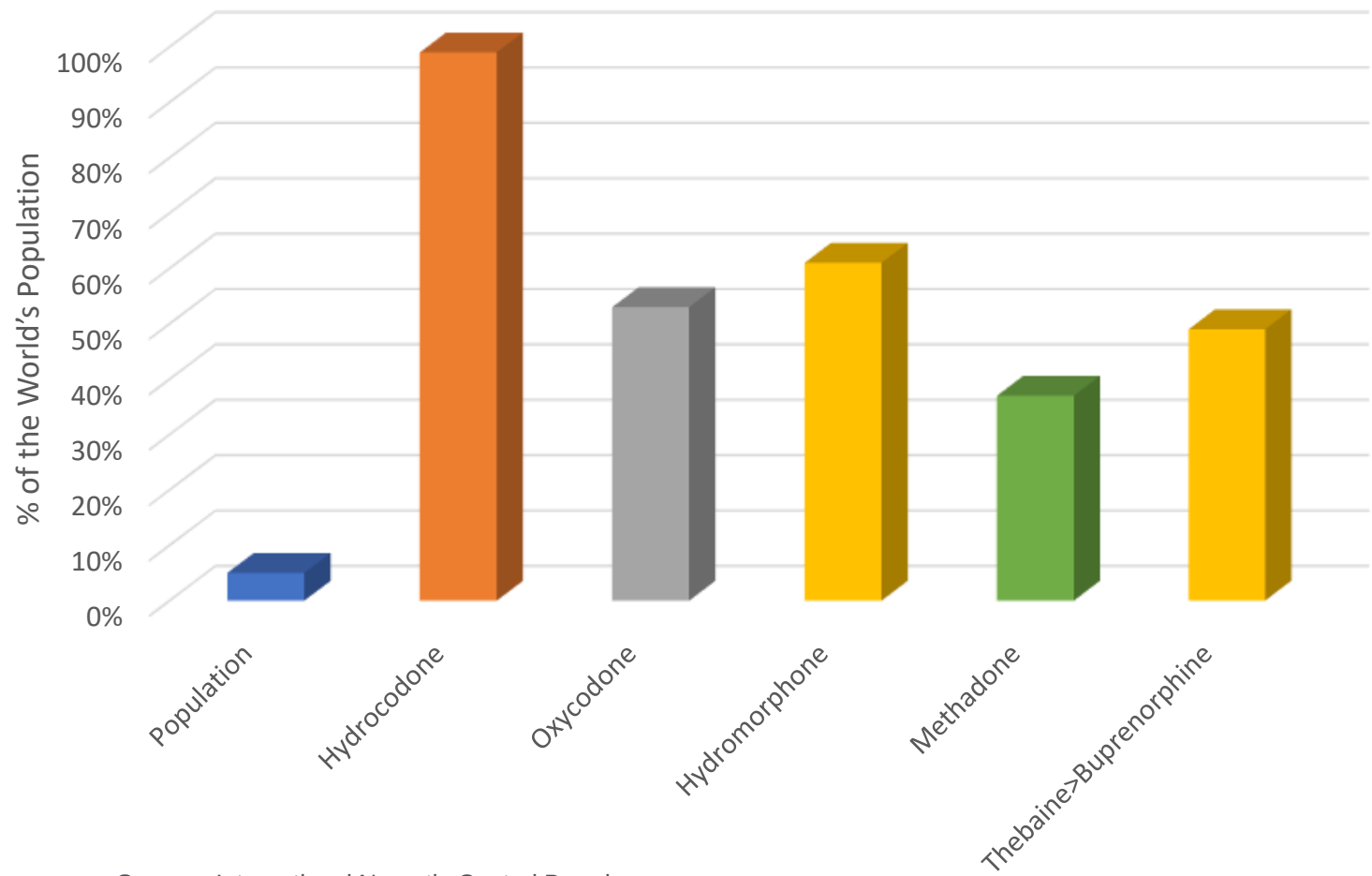
Describe problems and consequences of the **opioid crisis**.

Recognize **misprescribing and drug-seeking behaviors**.

Fulfill the State of Tennessee’s Board of Medical Examiner’s Continuing Medical education **requirement** of two hours every two years related to controlled substance prescribing including instruction in the Tennessee Chronic Pain Guidelines

USA's Appetite for Opioids

August 2018



Source: International Narcotic Control Board

Opioid Consumption



How did we
get here?

**ADDICTION RARE IN PATIENTS TREATED
WITH NARCOTICS**

To the Editor: Recently, we examined our current files to determine the incidence of narcotic addiction in 39,946 hospitalized medical patients¹ who were monitored consecutively. Although there were 11,882 patients who received at least one narcotic preparation, there were only four cases of reasonably well documented addiction in patients who had no history of addiction. The addiction was considered major in only one instance. The drugs implicated were meperidine in two patients,² Percodan in one, and hydromorphone in one. We conclude that despite widespread use of narcotic drugs in hospitals, the development of addiction is rare in medical patients with no history of addiction.

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1. Jick H, Miettinen OS, Shapiro S, Lewis GP, Siskind Y, Slone D. Comprehensive drug surveillance. *JAMA*. 1970; 213:1455-60.
2. Miller RR, Jick H. Clinical effects of meperidine in hospitalized medical patients. *J Clin Pharmacol*. 1978; 18:180-8.

17,000 U per square meter daily). Patients who had complete remissions (except for three over 60 years of age) received central-nervous-system therapy (2400 rads to the skull, with five intrathecal injections of methotrexate or arabinosyl cytosine, or both). During complete remission, they were given 6-mercaptopurine (70 mg per square meter daily), methotrexate (25 mg per square meter each week), and courses of vincristine and prednisone every three to four months.

Results are shown in Table 1. They do not support the suggestion by Dr. Bitran that in adults with acute lymphoblastic anemia, T-cell leukemia has a poorer prognosis than B-cell disease. However, because of the limited number of cases and the short follow-up, the present data are far from definitive. More information on this point is needed. The identification of prognostic factors in acute lymphoblastic anemia in adults is critical, not only for the choice of induction therapy but also because young adults with an established poor prognosis could profit from allogeneic-marrow transplantation during the first remission. Therefore, we suggest that for the time being it may be wiser to base prognosis on more established criteria, such as age and blast-cell count in the blood.²

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1. Bitran JD. Prognostic value of immunologic markers in adults with acute lymphoblastic leukemia. *N Engl J Med*. 1978; 299:1317.

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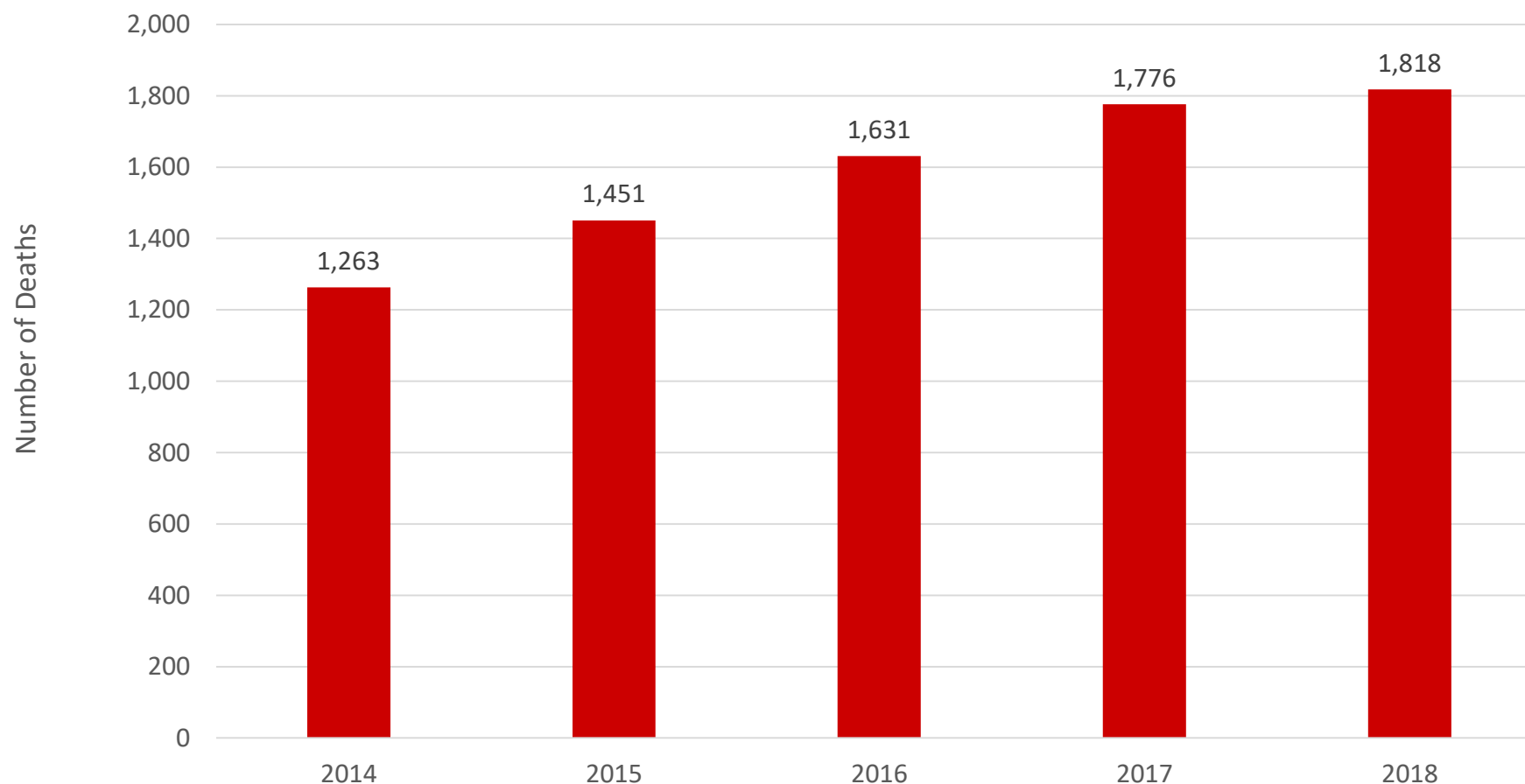
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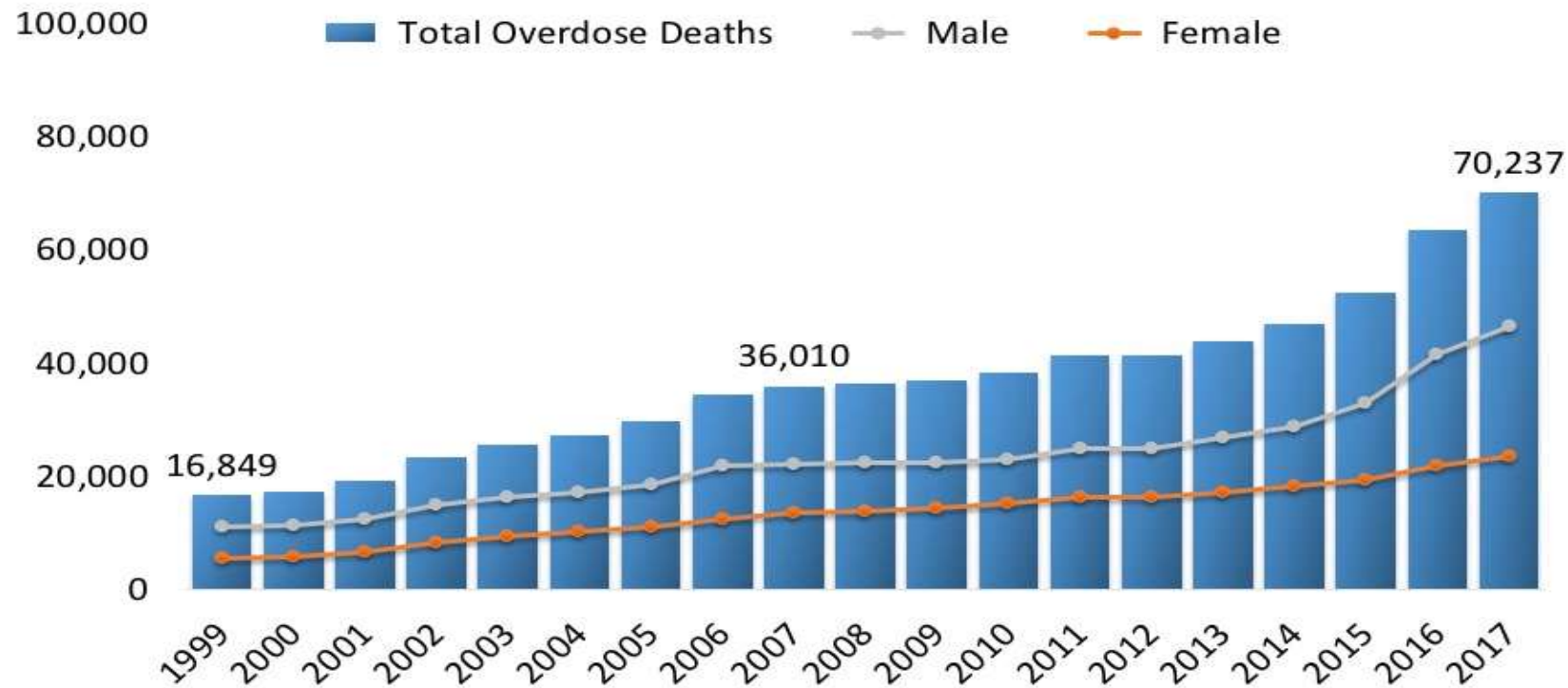
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Drug Overdose Deaths in Tennessee



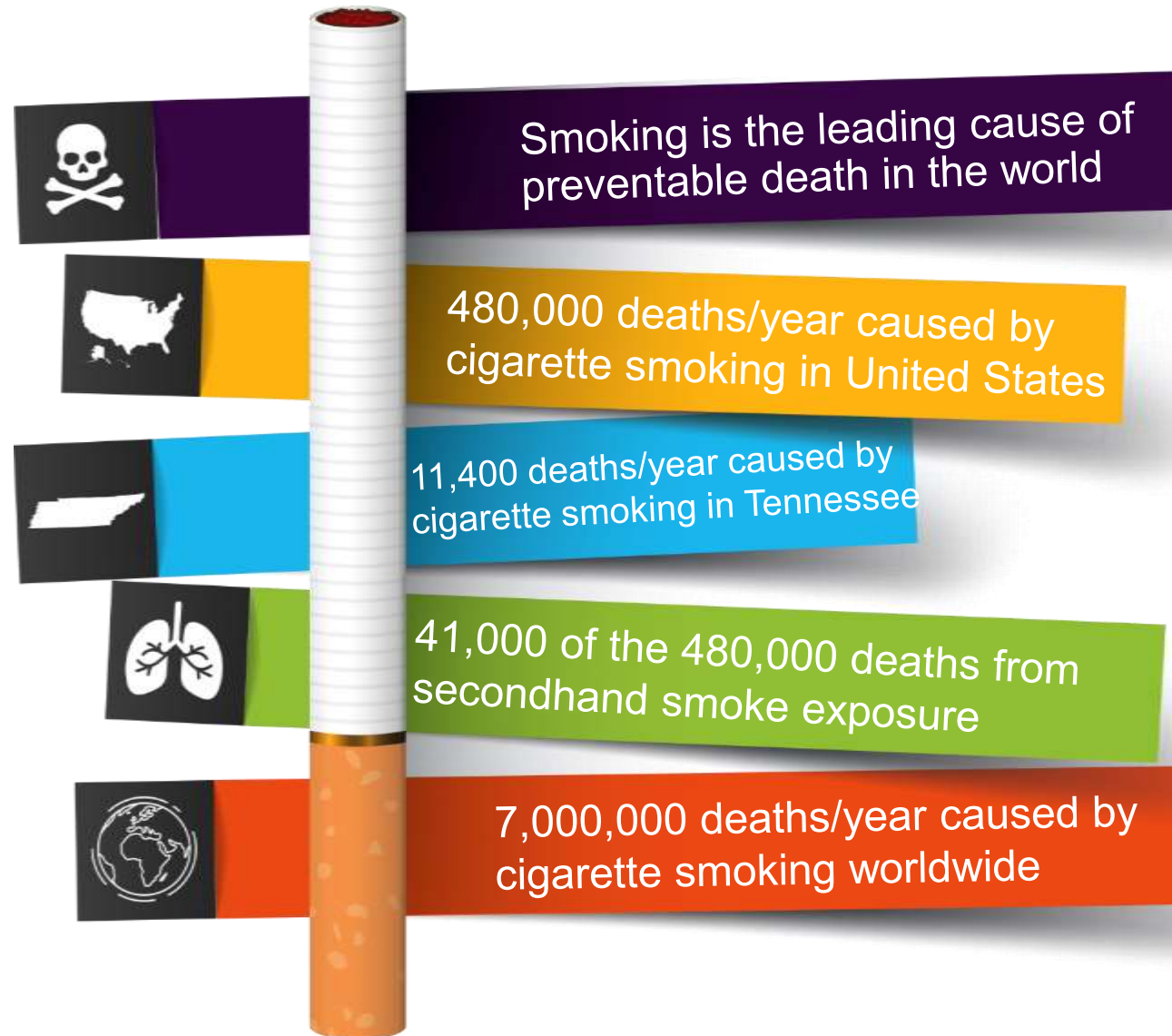
Source: Tennessee Department of Health, Office of Informatics and Analytics

Figure 1. **National Drug Overdose Deaths**
Number Among All Ages, by Gender, 1999-2017

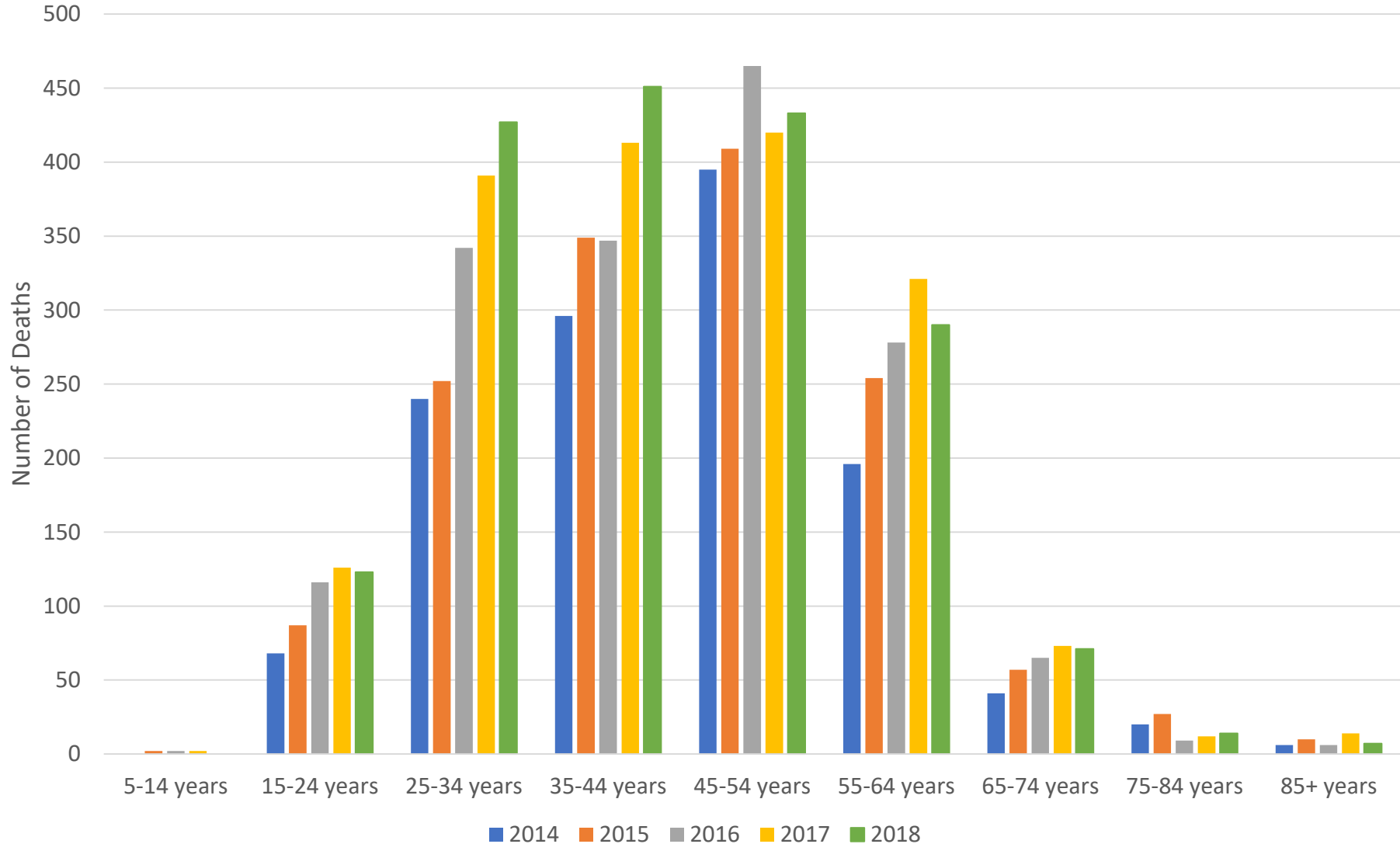


Source: : Centers for Disease Control and Prevention, National Center for Health Statistics. Multiple Cause of Death 1999-2017 on CDC WONDER Online Database, released December, 2018

Compared to Opioids, Nicotine...

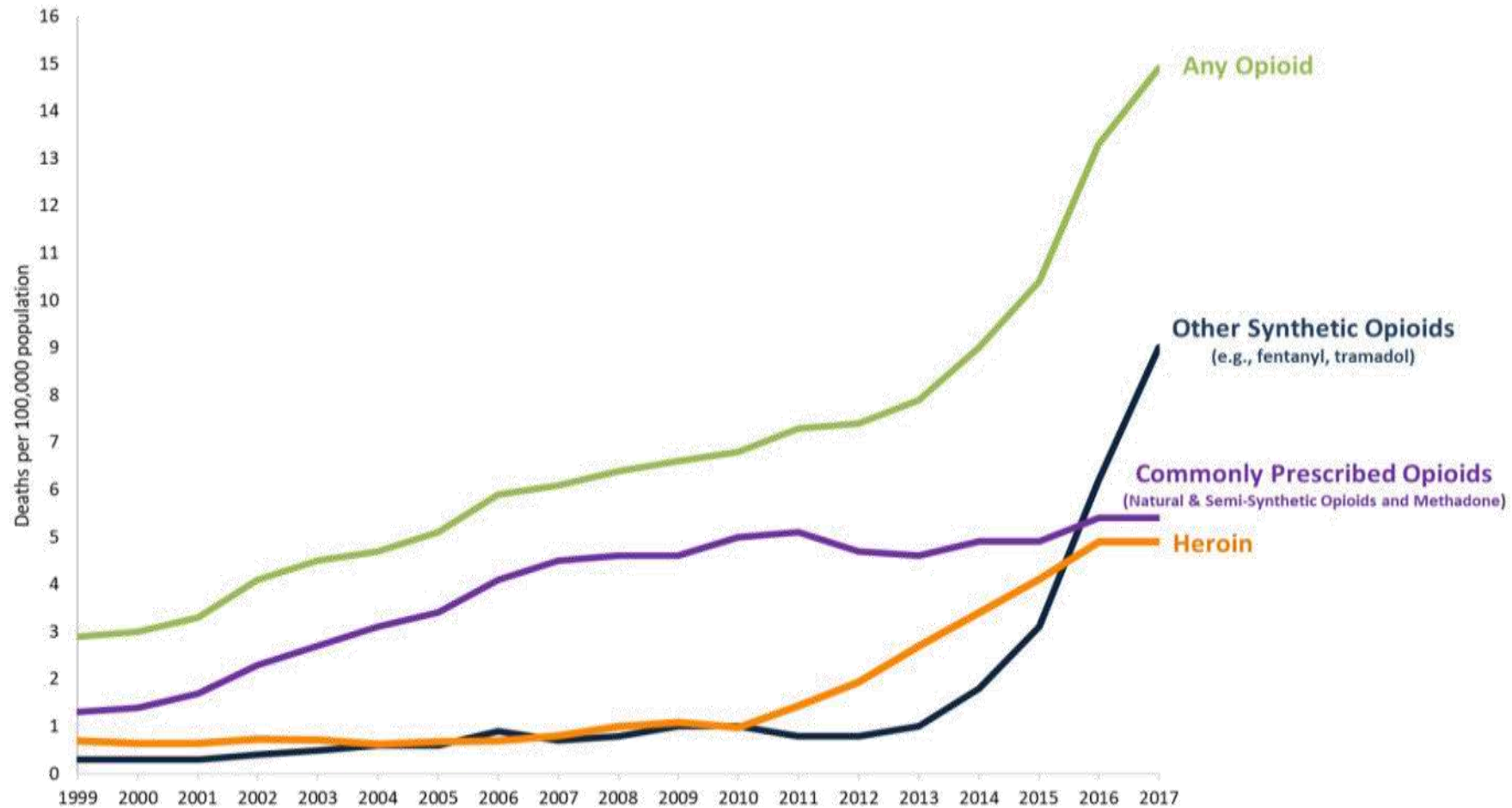


Tennessee Overdose Deaths by Age



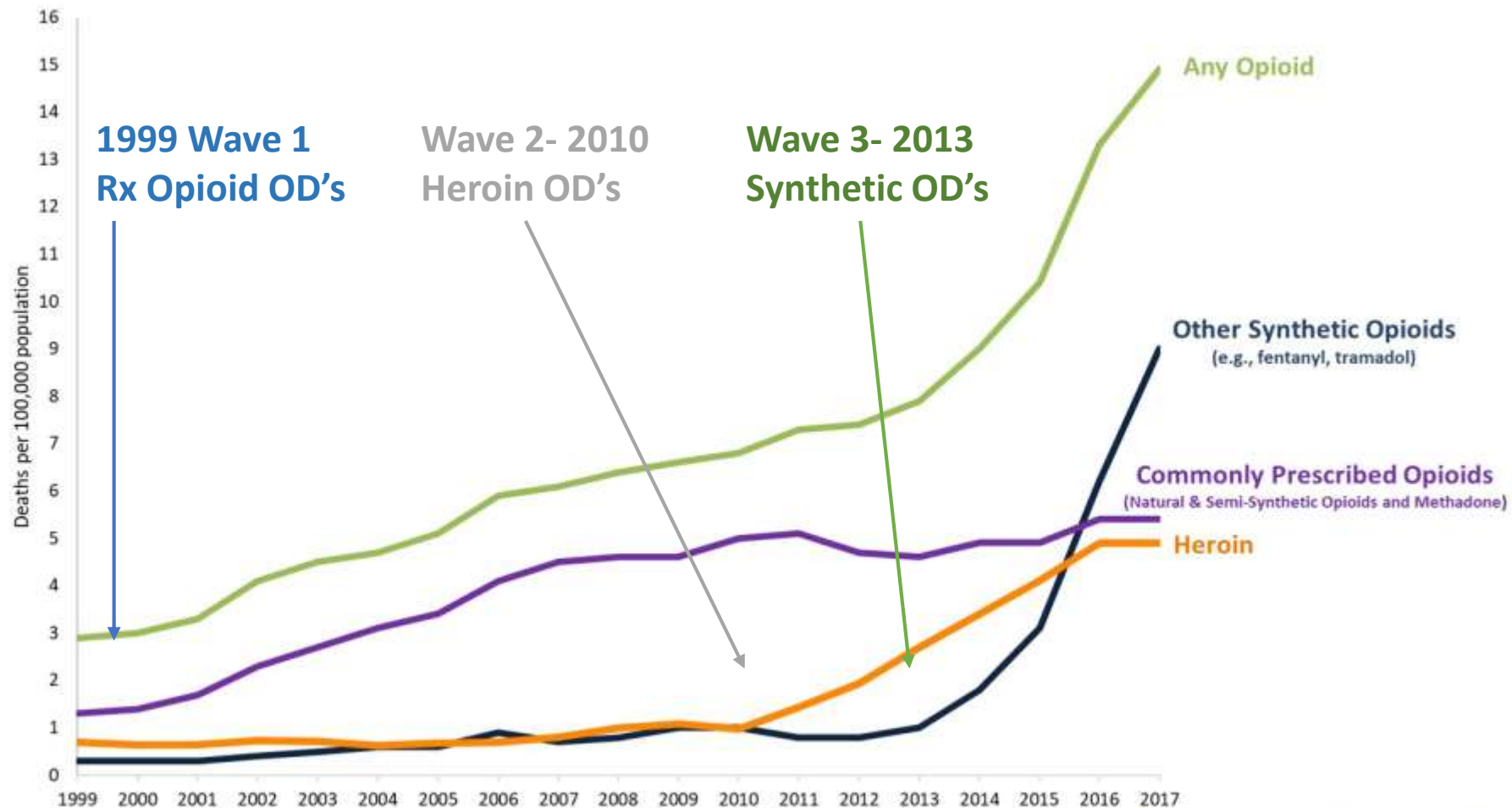
Source: Tennessee Department of Health, Office of Informatics and Analytics

Overdose Death Rates Involving Opioids, by type, United States, 1999-2017



Source: CDC/NCHS, National Vital Statistics System, Mortality. CDC WONDER, Atlanta, GA: US Department of Health and Human Services, CDC; 2018. <https://wonder.cdc.gov/>.

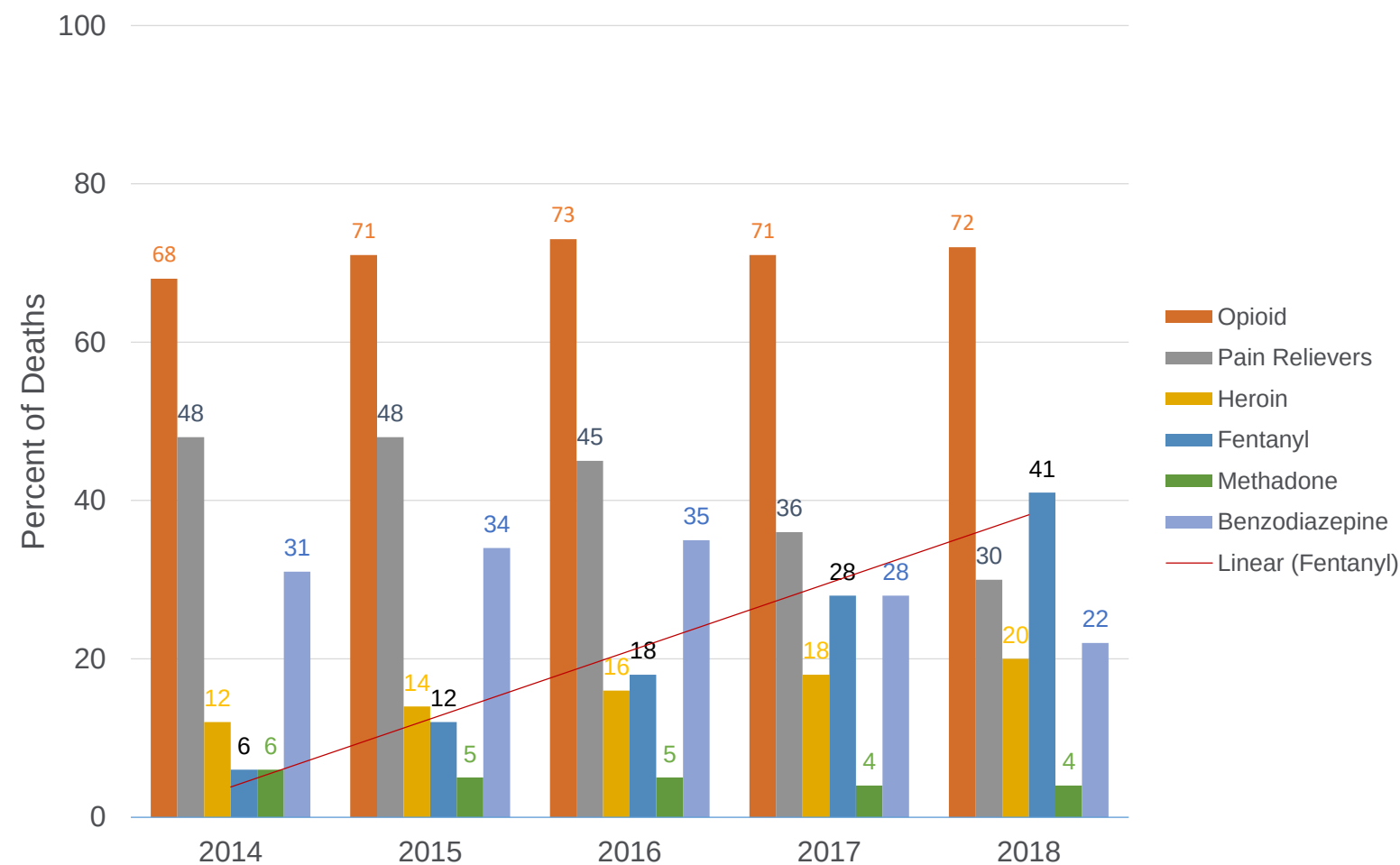
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SOURCE: CDC/NCHS, National Vital Statistics System, Mortality. CDC WONDER, Atlanta, GA: US Department of Health and Human Services, CDC; 2018.
<https://wonder.cdc.gov/>.

www.cdc.gov
Your Source for Credible Health Information

Tennessee Trends in Overdose Deaths by Drug Type



Source: Tennessee Department of Health, Office of Informatics and Analytics

Dangers of Fentanyl



Direct substitute for heroin or opioid addiction

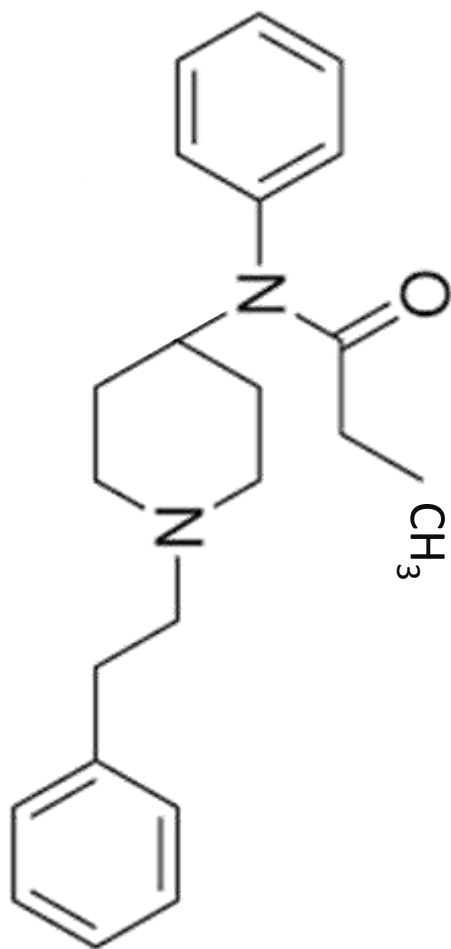
Dose required for euphoric effect also induces respiratory depression

Requires an “accurate cut” which is almost impossible clandestinely

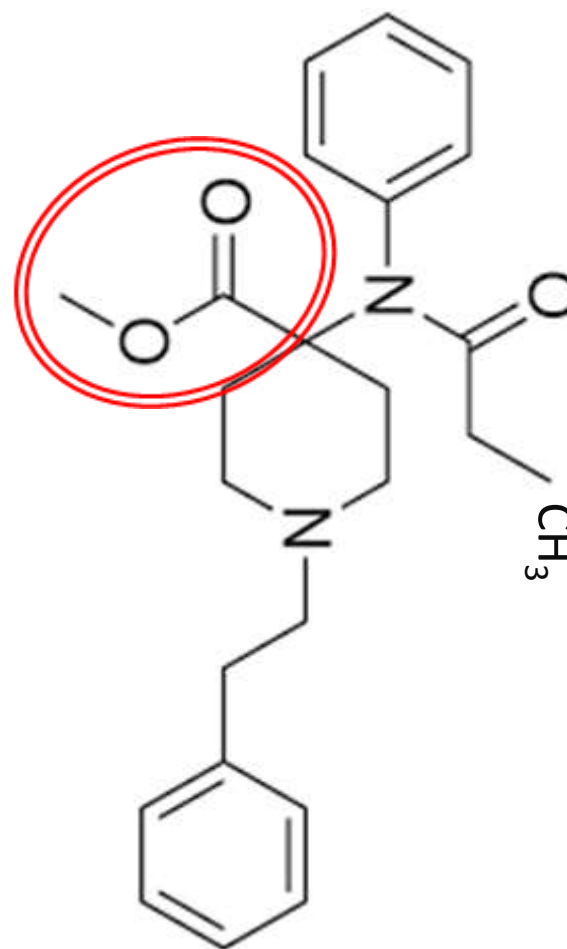
Utilized as cutting agent for heroin, methamphetamine, cocaine, and counterfeit

Physical characteristics present significant hazards

FENTANYL



CARFENTANIL



Lethal Doses of Fentanyl & Carfentanil



- Fatal dose of Fentanyl
- 2 mg or 2000 mcg



- Fatal dose of Carfentanil
- 0.02 mg or 20 mcg

Alprazolam or Fentanyl laced Alprazolam



Counterfeit Fentanyl Laced



Genuine Alprazolam

Oxycodone or Fentanyl laced Oxycodone



Counterfeit Fentanyl Laced



Oxycodone 30 mg

Mobile Pill Manufacturing Plant





CLINICAL



VIGNETTE



For every Vignette think...



Is this good medicine or not?



Is this:

- Mis-prescribing?
- Aberrant behavior?
- Diversion?
- Addiction?



What do I need to change in my practice to prevent this?

CLINICAL VIGNETTE

- Mis-prescribing: Prescribing Controlled Prescription Drugs in quantities and/or frequency inappropriate for the patient's complaint or illness.
- Aberrant Behavior: Behaviors that indicate misuse of a prescribed medication or addiction.
- Addiction: Continued use of a substance or process despite consequences

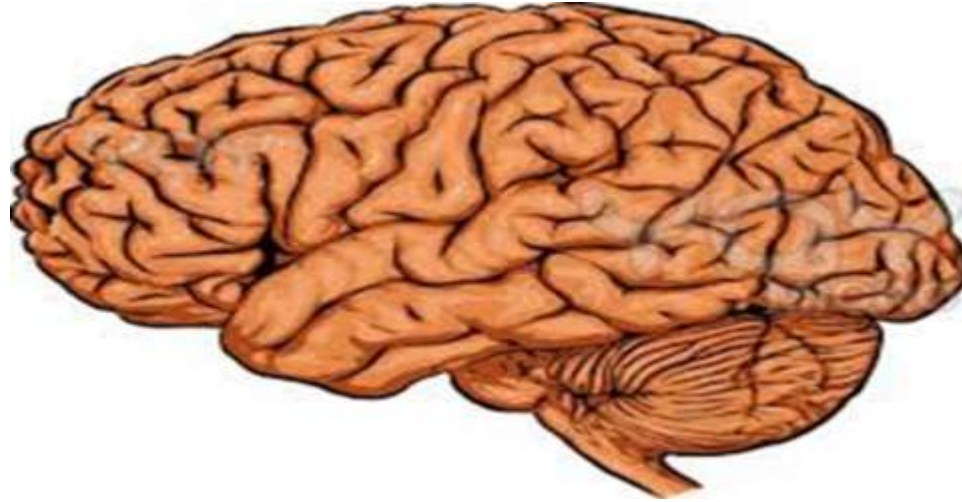
CLINICAL VIGNETTE

Mr. Mel A. Noma, a 68-year-old fair skinned man sees his dermatologist for his annual visit. He is otherwise healthy, treated only for migraine headaches with a triptan every so often. For the last 25 years he has undergone skin biopsies every 2-3 years and has received a Rx after each procedure for Oxycodone/APAP 5/500mg, #15.  As a dutiful patient he filled the Rx's. He has never needed or taken a pain pill and has a collection in the medicine cabinet. 

CLINICAL VIGNETTE

- What are the red flags?
 1. No questions about post-op pain history.
 2. Received and filled an Rx that was not needed.
 - 3. Standing Opioid Rx orders are risky.**
 4. His granddaughter took the 140 opioid pills that were in the medicine cabinet. She had a fatal overdose while with 2 of her friends at a “Fish Bowl” party

What is Addiction?



Addiction is a brain disease

Addiction is the single most preventable cause of
Disability and Death in the U.S.

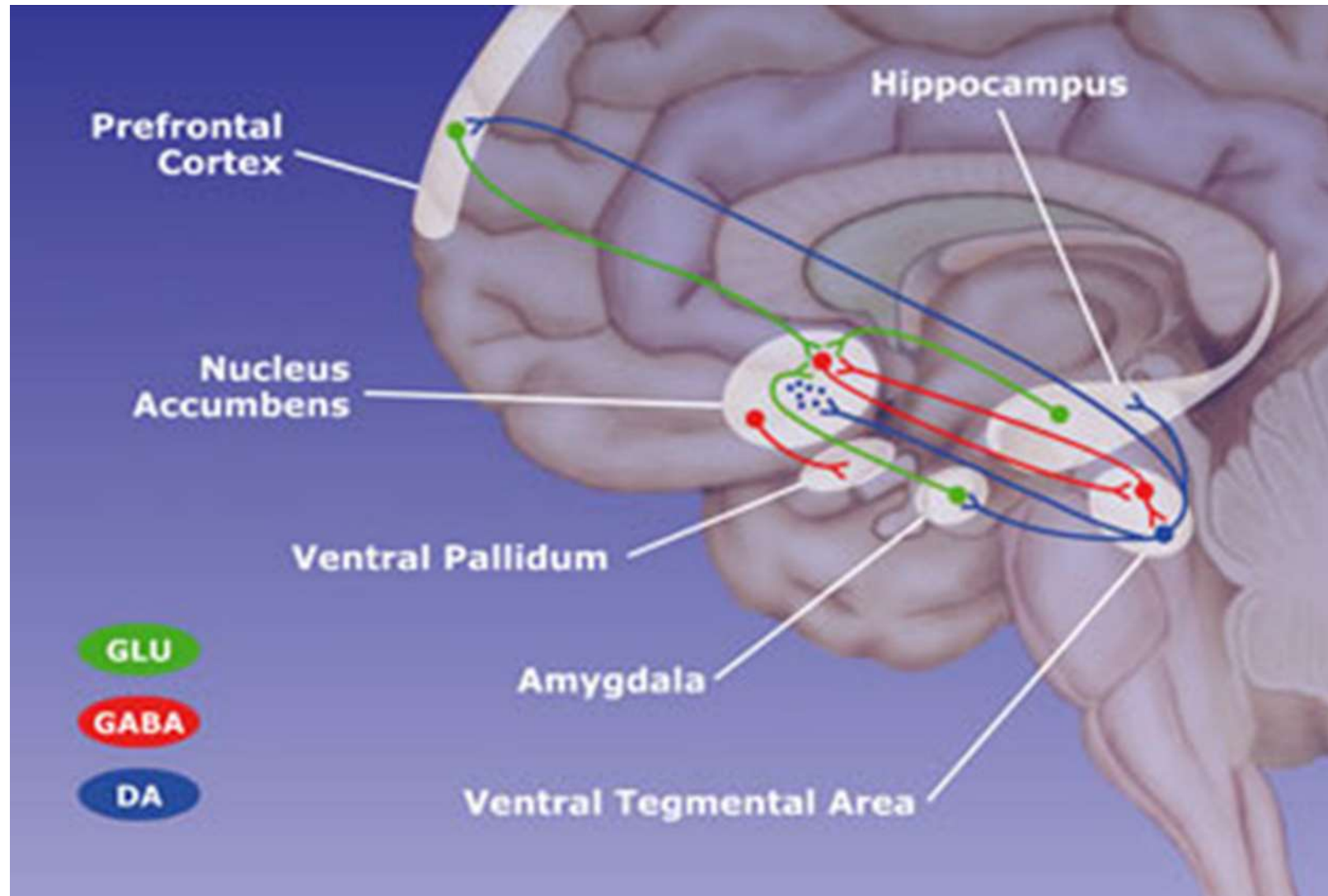
Active Addiction Feels Like



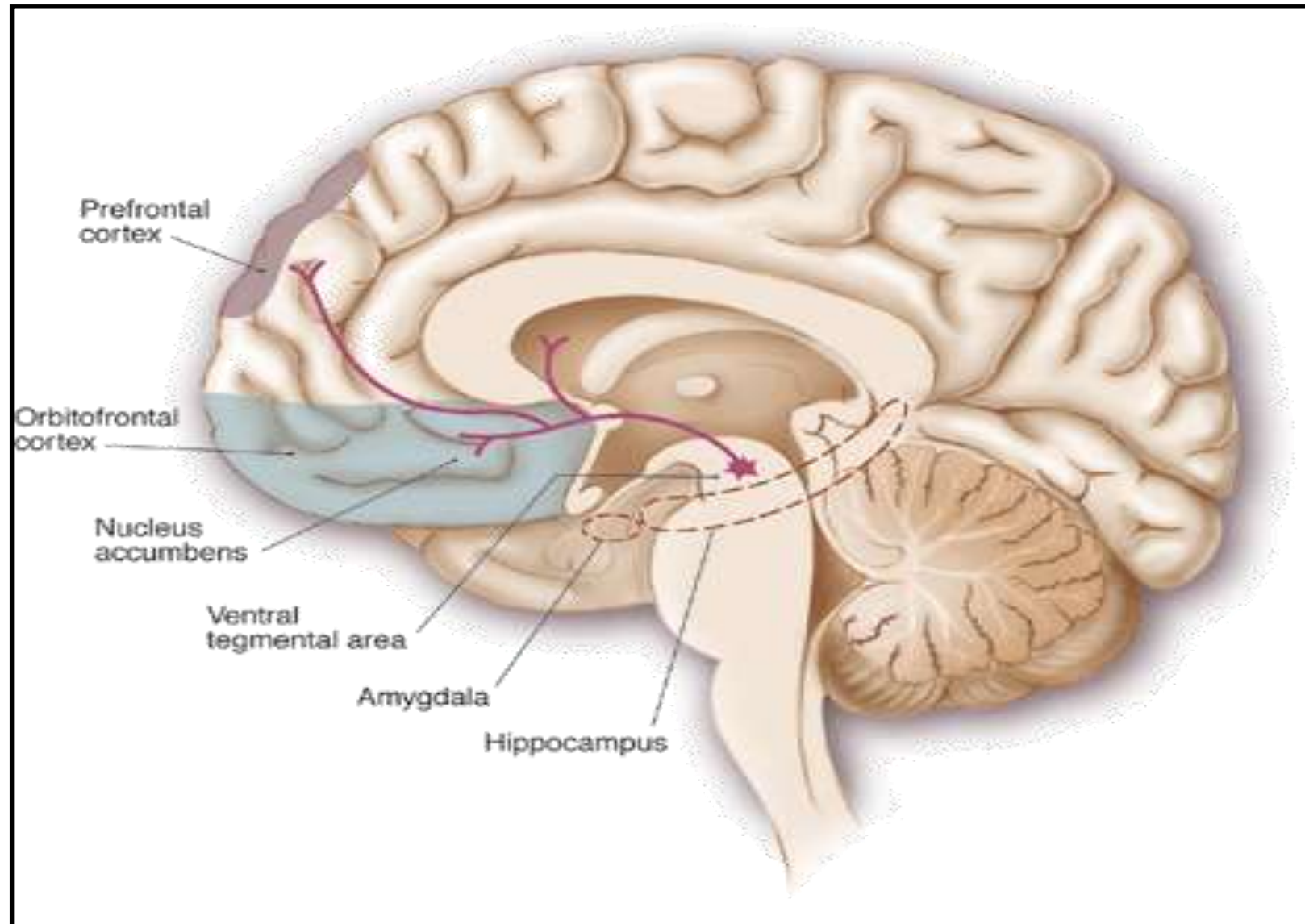
“Addiction doesn’t come heralded by a brass band, it sneaks up on you, and sometimes with extraordinary speed.”

C. Everett Koop (former US Surgeon General), 2003

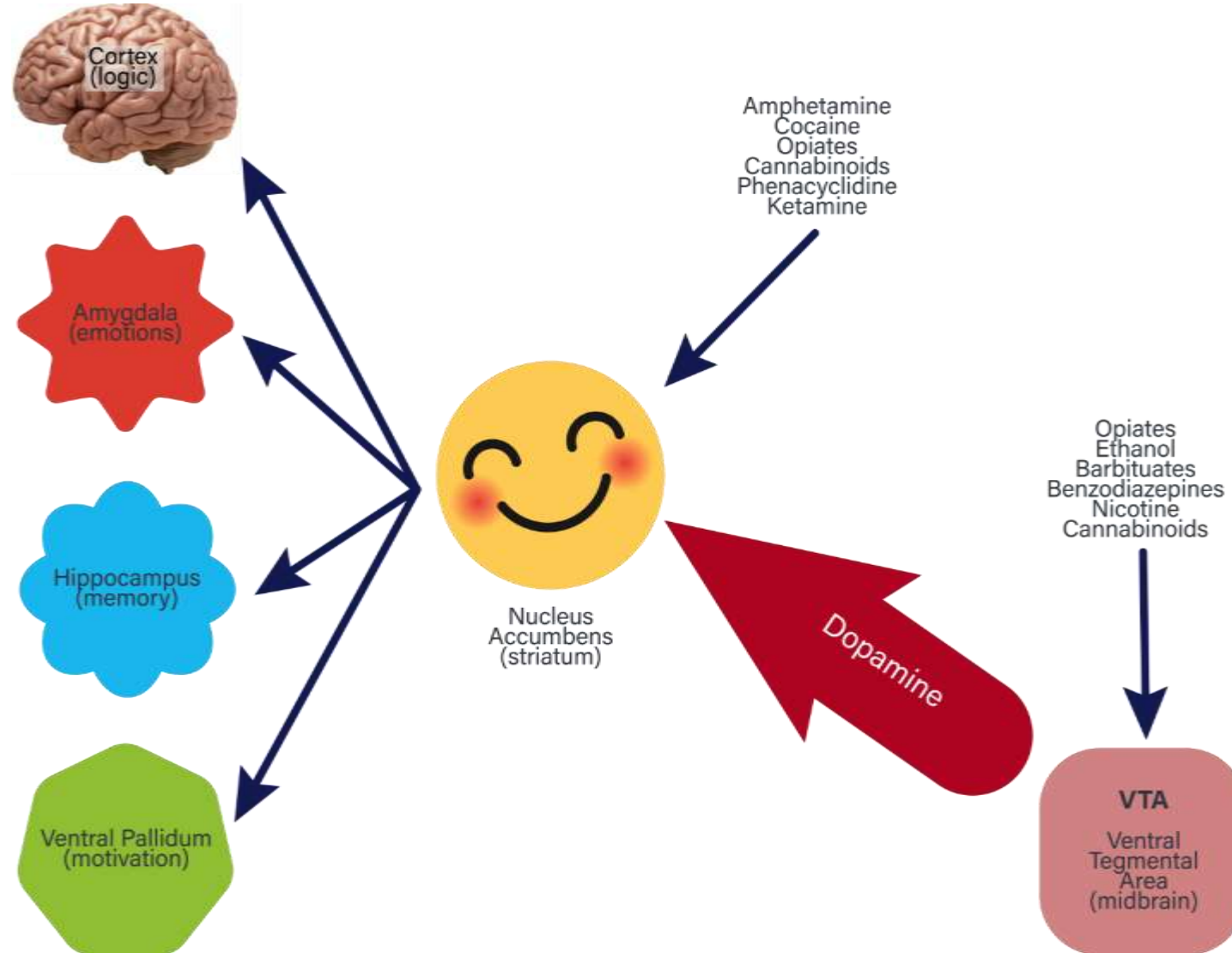
The Reward Circuit



The Reward Center



The Pleasure Circuit



Addiction = Substance Use Disorder

NIDA

Addiction

A state in which an organism engages in a compulsive behavior

- Behavior is reinforcing (rewarding or pleasurable)
- Loss of control in limiting intake

ASAM

Addiction is a primary, chronic, neurobiologic disease with genetic, psychosocial, and environmental factors influencing its development and manifestations.

It is characterized by behaviors that include one or more of the following “**4 C's**”.

- Impaired **Control** over drug use.
- **Compulsive** use.
- **Continued** use despite harm.
- **Craving**

Substance Use Disorder = Addiction

DSM-IV

- Abuse
Willful misuse
- Dependence
Addiction

DSM-5

- Substance Use Disorder

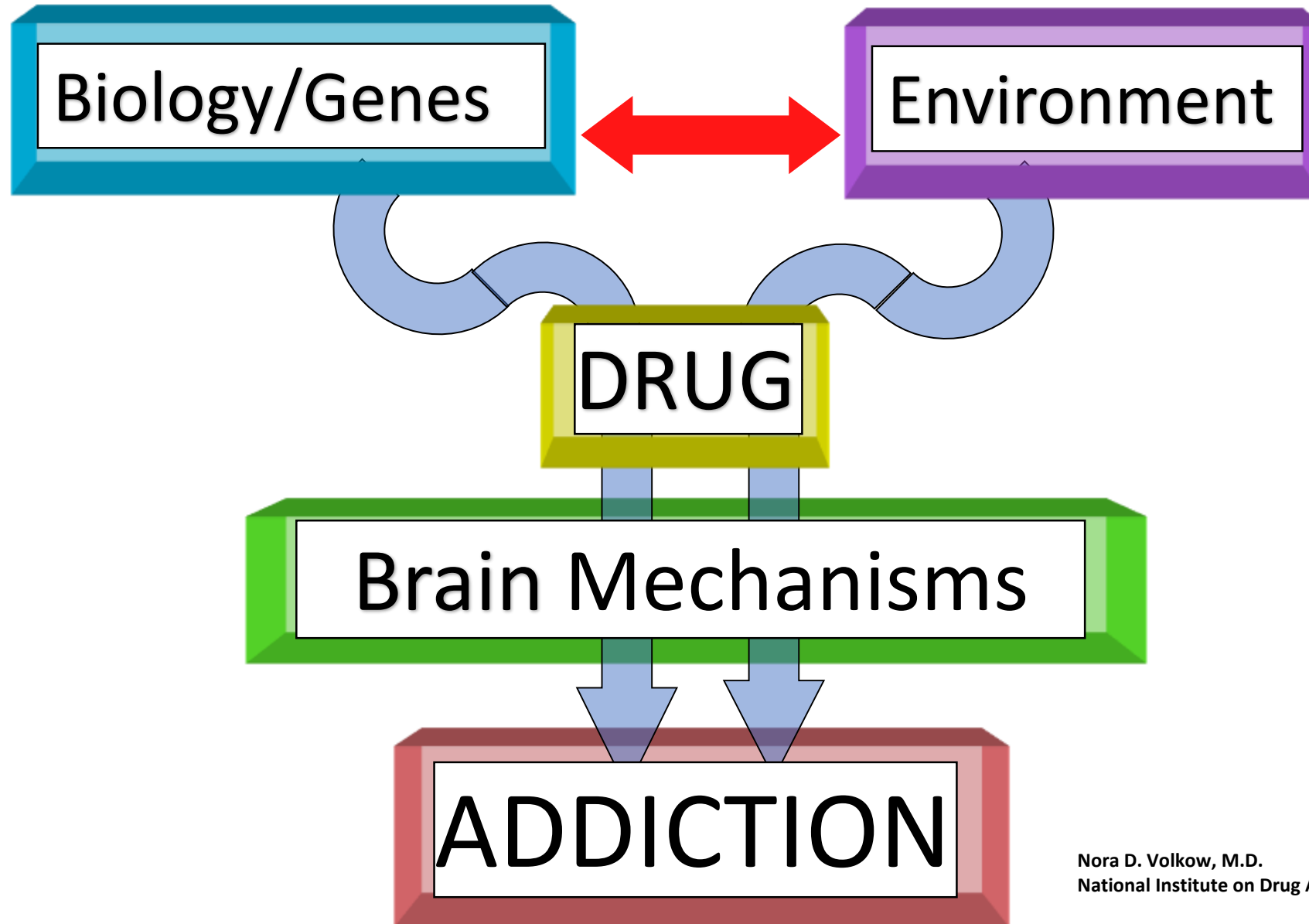
Recurrent use of alcohol or other drugs that causes clinically and functionally significant impairment, such as health problems, disability, and failure to meet major responsibilities at work, school, or home.

Depending on the level of severity, this disorder is classified as mild, moderate, or severe

	DSM-IV Abuse	DSM-IV Dependence	DSM-5 Substance Use Disorders
Hazardous Use	◆	-	◆
Social/interpersonal problems related to use	◆	-	◆
Neglected major roles due to use	◆	-	◆
Legal problems	◆	-	◆
	} ≥1 criterion		} ≥2 criteria
Withdrawal	-	◆	
Tolerance	-	◆	
Used larger amounts longer	-	◆	
Repeated attempts to quit/control use	-	◆	
Much time spent using	-	◆	
Physical/psychological problems related to use	-	◆	
Activities given up to use	-	◆	
			} ≥3 criteria
Craving	-	-	◆

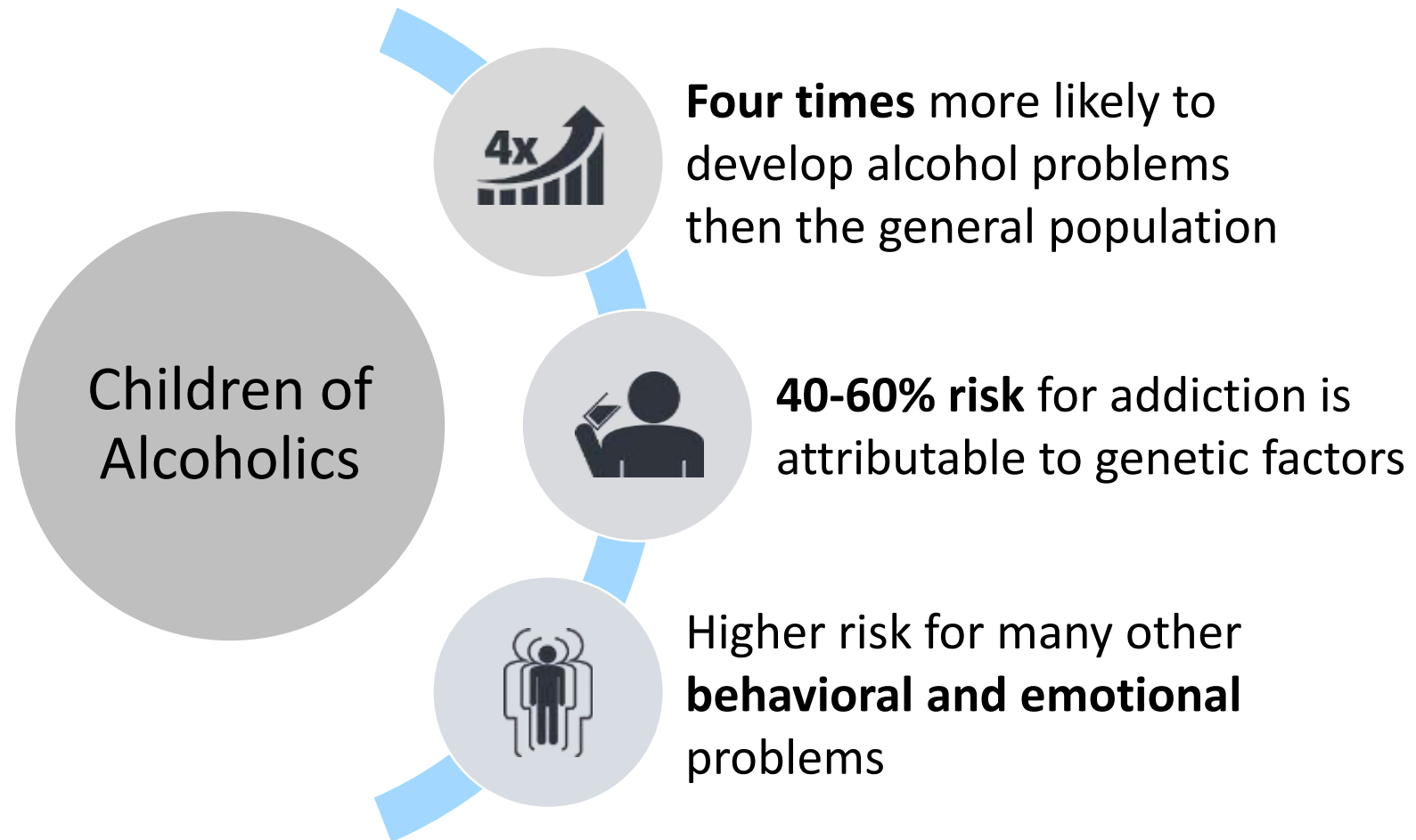
DSM-5 Criteria	
2	Mild SUD
3	
4	Moderate SUD
5	
≥ 6	Severe SUD

Addiction



Nora D. Volkow, M.D.
National Institute on Drug Abuse

Etiology - Genetics

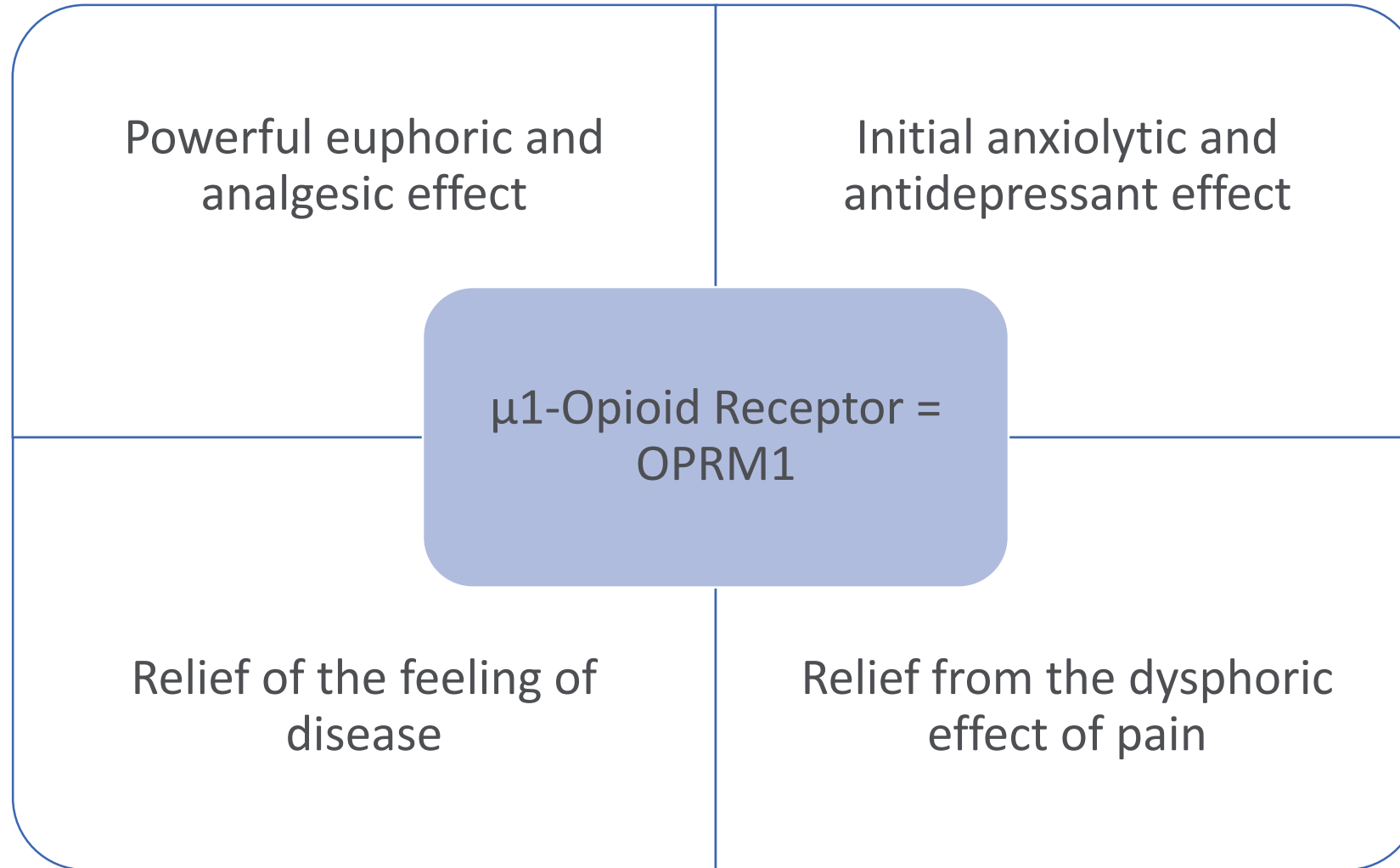


Etiology - Genetics

Genetic and environmental influences may be correlated to substance initiation and use.

Progression to Addiction (Substance Use Disorder) is more related to genetic factors

Etiology - Genetics



Candidate Genes for Substance Abuse

Table 1. Genes having one or more variants that have been reported to be associated with one or more addictions.

Gene	System	Protein	Chromosomal location	Drug
OPRM1	Opioid	μ opioid receptor	6q24-25	Heroin/opiate; Alcohol
OPRK1	Opioid	κ opioid receptor	8q11.2	Heroin/opiate
PDYN	Opioid	Preprodynorphin	20pter-p12.2	Cocaine/stimulants
TH	Dopaminergic	Tyrosine Hydroxylase	11p15.5	Alcohol
DRD2	Dopaminergic	Dopamine receptor 2	11q23	Alcohol
DRD3	Dopaminergic	Dopamine receptor 3	3q13.3	Alcohol
DRD4	Dopaminergic	Dopamine receptor 4	11p15.5	Heroin/opiate; Cocaine/stimulants; Alcohol
DBH	Dopaminergic	Dopamine β -hydroxylase	9q34	Cocaine/stimulants
DAT	Dopaminergic	Dopamine transporter	5p15.3	Alcohol
TPH1	Serotonergic	Tryptophan hydroxylase 1	11p15.3-p14	Alcohol
TPH2	Serotonergic	Tryptophan hydroxylase 2	12q21.1	Heroin/opiate; Alcohol
HTR1B	Serotonergic	Serotonin receptor 1B	6q13	Heroin/opiate; Alcohol
HTR2A	Serotonergic	Serotonin receptor 2A	13q14-q21	Alcohol
SERT	Serotonergic	Serotonin transporter	17q11.1-q12	Heroin/opiate; Alcohol
MAOA	Catecholaminergic/Serotonergic	Monoamine oxidase A	Xp11.23	Alcohol
COMT	Catecholaminergic	Catechol-O-methyl transferase	22q11.2	Heroin/opiate; Alcohol
GABRA1	GABAergic	GABA receptor subunit α -1	5q34-q35	Alcohol
GABRA6	GABAergic	GABA receptor subunit α -6	5q31.1-q35	Alcohol
GABRB1	GABAergic	GABA receptor subunit β -1	4p13-p12	Alcohol
CHRM2	Cholinergic	Muscarinic acetylcholine	7q35-q36	Alcohol
CNR1	Cannabinoid	Cannabinoid receptor 1	6q14-q15	Cocaine/stimulants; Alcohol
FAAH	Cannabinoid	Fatty acid amide hydrolase	1p35-34	Alcohol
NPY	Neuromodulatory	Neuropeptide Y	7p15.1	Alcohol
ADH1B	Ethanol Metabolism	Alcohol dehydrogenase 1B	4q22	Alcohol
ADH1C	Ethanol Metabolism	Alcohol dehydrogenase 1C	4q22	Alcohol
ALDH2	Ethanol metabolism	Alcohol dehydrogenase 2	12q24.2	Alcohol
CYP2D6	Drug metabolism	Cytochrome CYP450	22q18.1	Heroin/opiate
ANKK1	Signal transduction	Ankyrin repeat and kinase domain-containing 1	11q23.2	Alcohol

Source: Kreek, et al, Nature, December 2005.

Etiology – Adolescent Traits

High novelty and
sensation seeking

Impulsivity

Low harm
avoidance

High reward
dependence

Temperament –
anxiety,
aggression, or
irritability

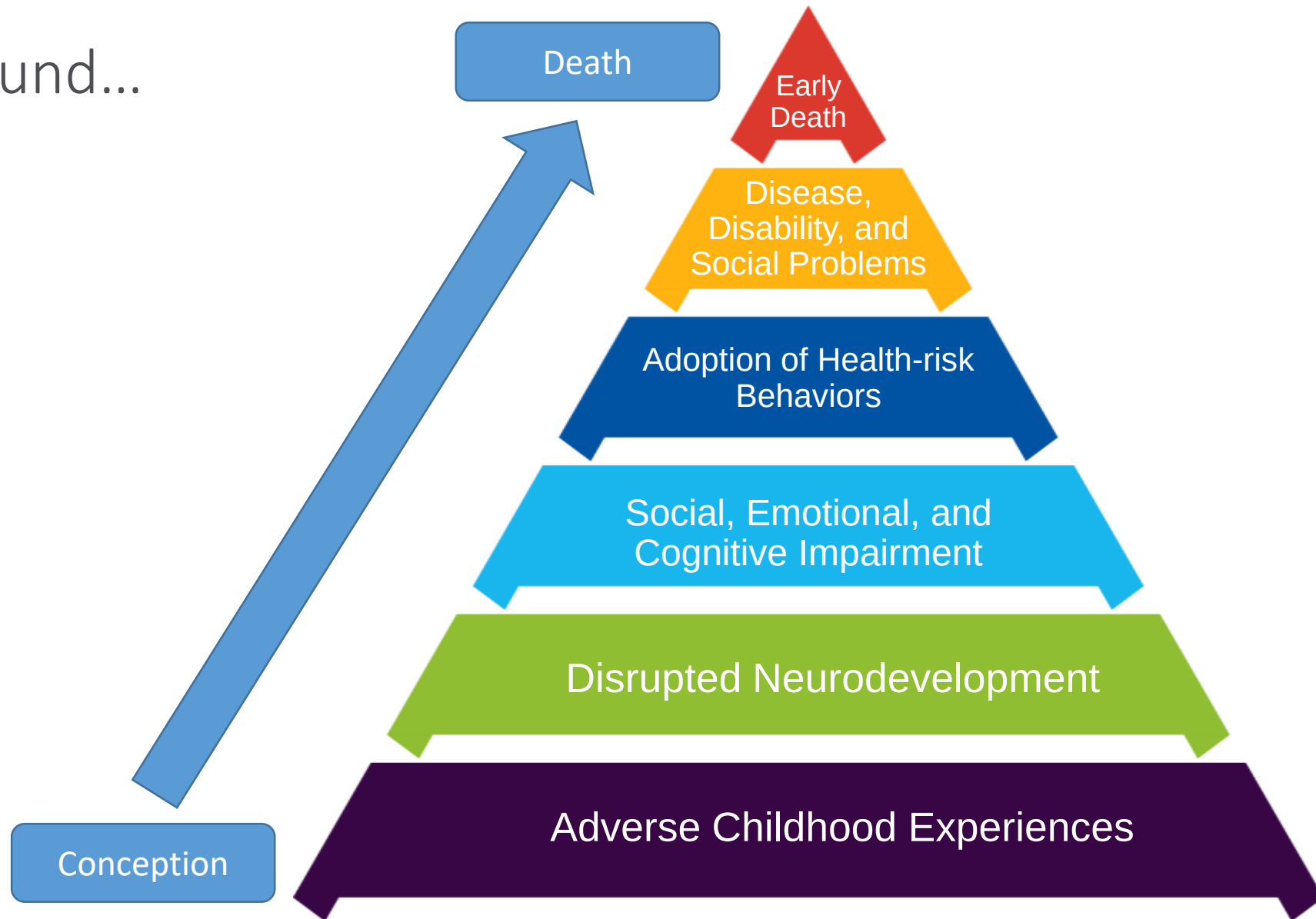
Intelligence –
cognitive abilities

Etiology – Adverse Childhood Experiences (ACE)

The quantity of traumatic events experienced in childhood correlates with the development of addiction.

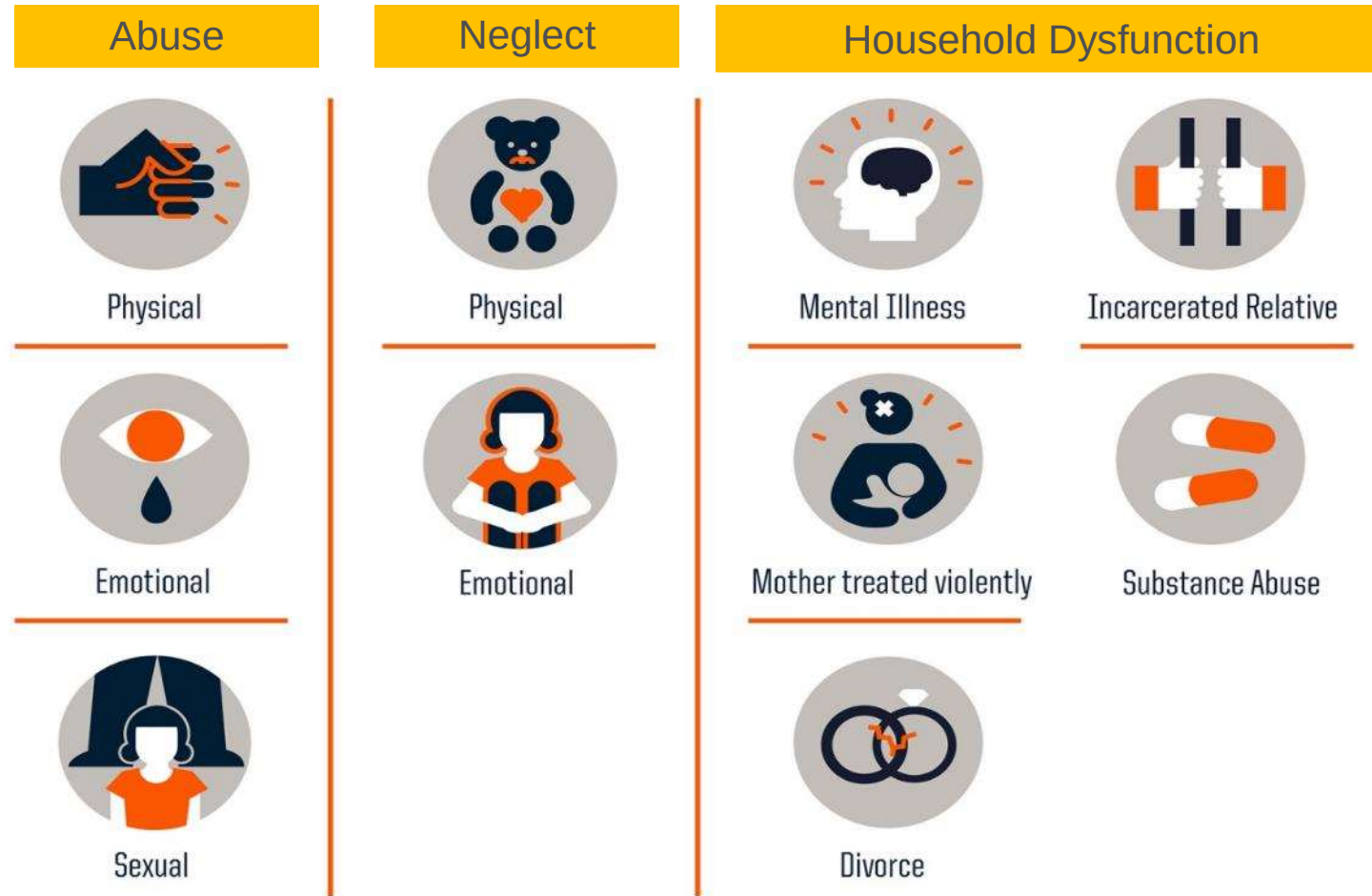
The greater the number the more likely the development of addiction

ACE Found...

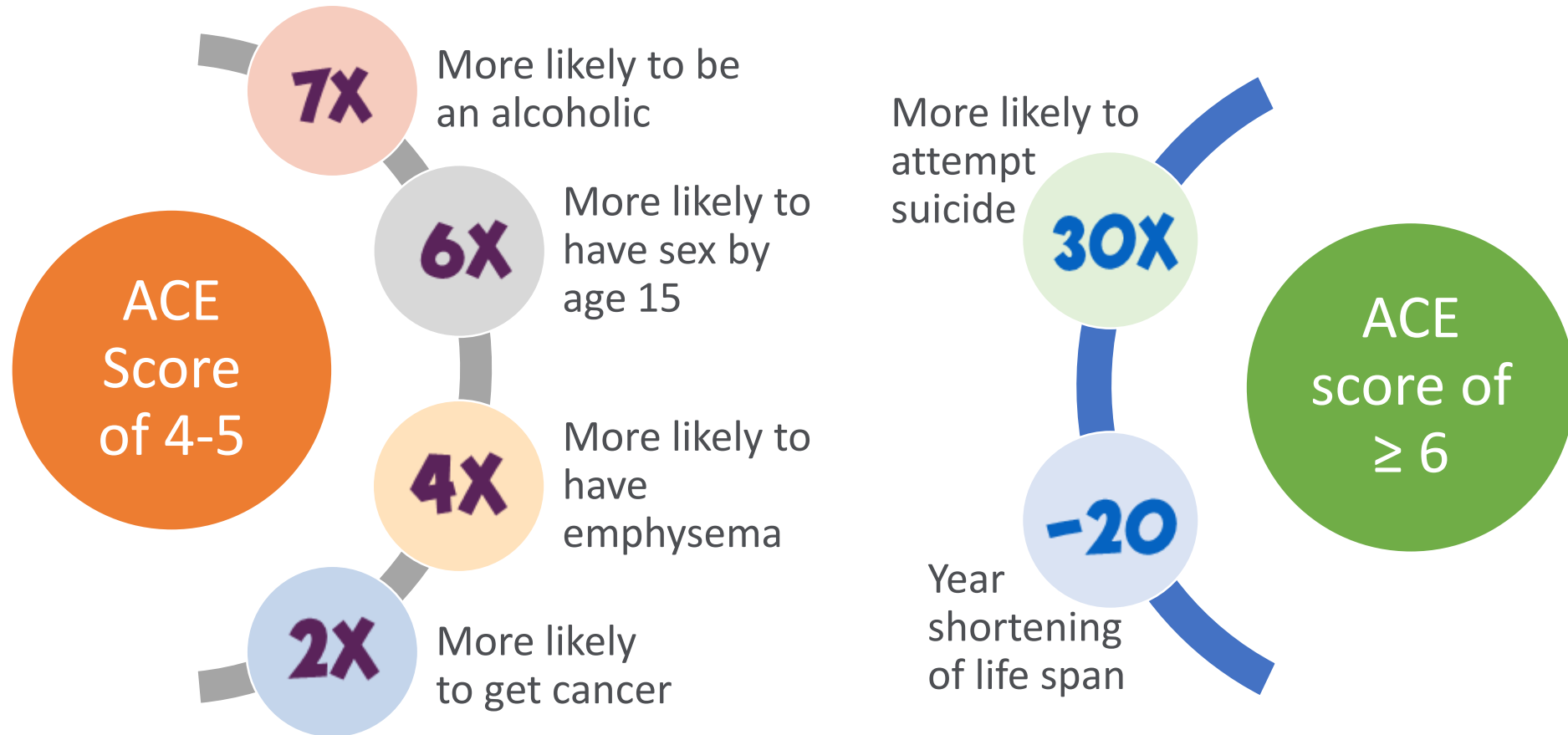


Mechanisms by Which Adverse Childhood Experiences Influence Health and Well-being Throughout the Lifespan

What Does ACE Look Like?



ACE Score Predictability



High ACE Score Correlates With ...



Substance use
disorder/addiction



Heart disease



Anxiety disorders



Obesity



Depression



Suicide attempts



Diabetes



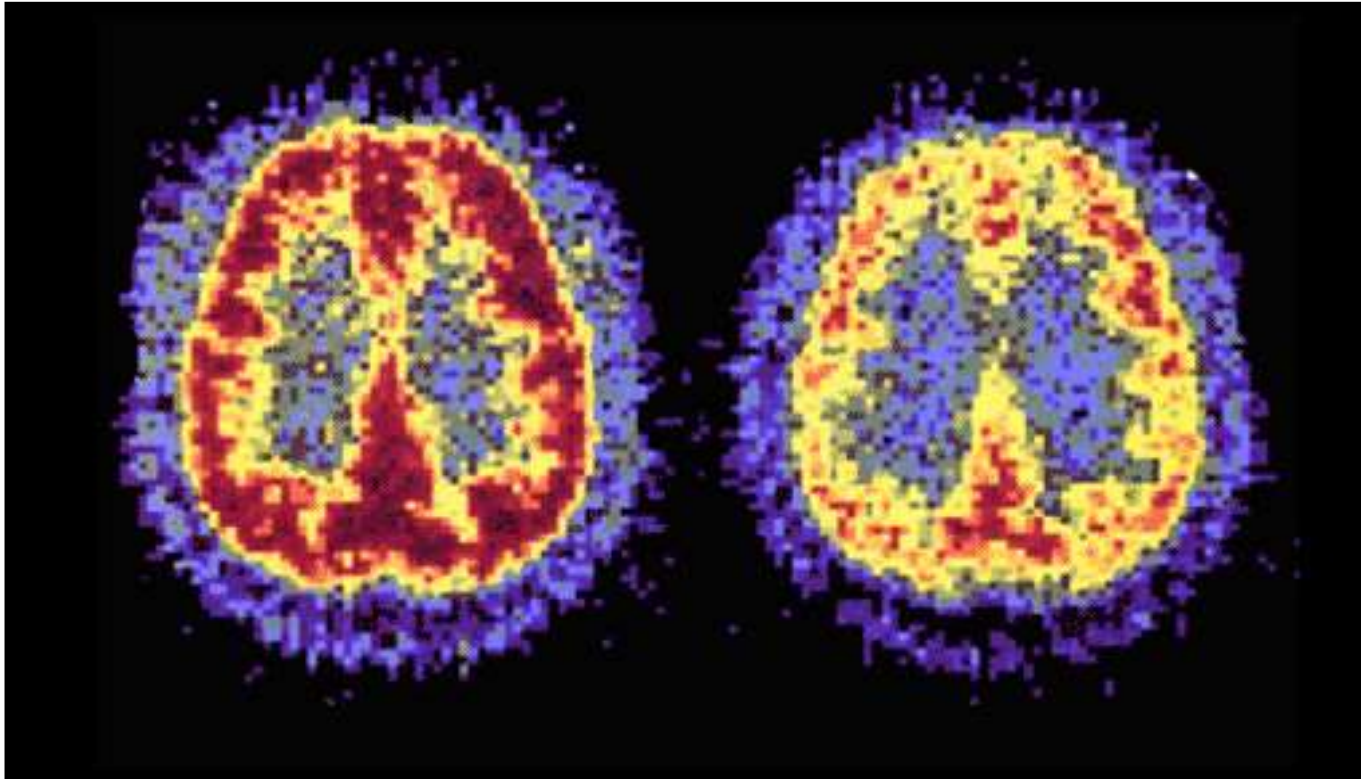
Increased risk for
intimate partner
violence

Etiology - Exposure

Postsurgical prescriptions for opioid naive patients and association with overdose and misuse: retrospective cohort study

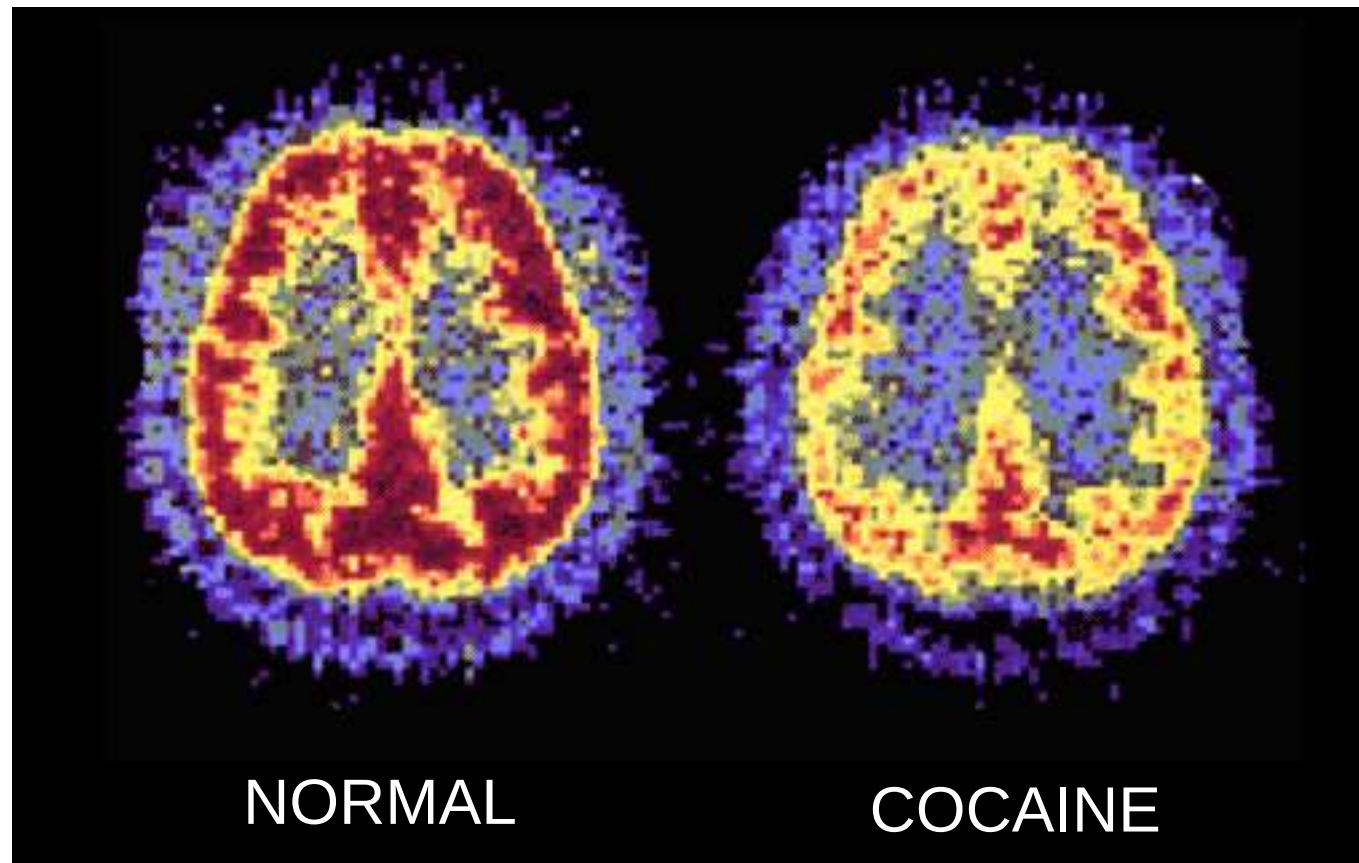
- 568,612 patients (56% of N) received postoperative opioids.
- Total duration of opioid use was the strongest predictor of misuse.
- A single refill increased the potential of misuse by more than 40%

Which Brain Is on Drugs?

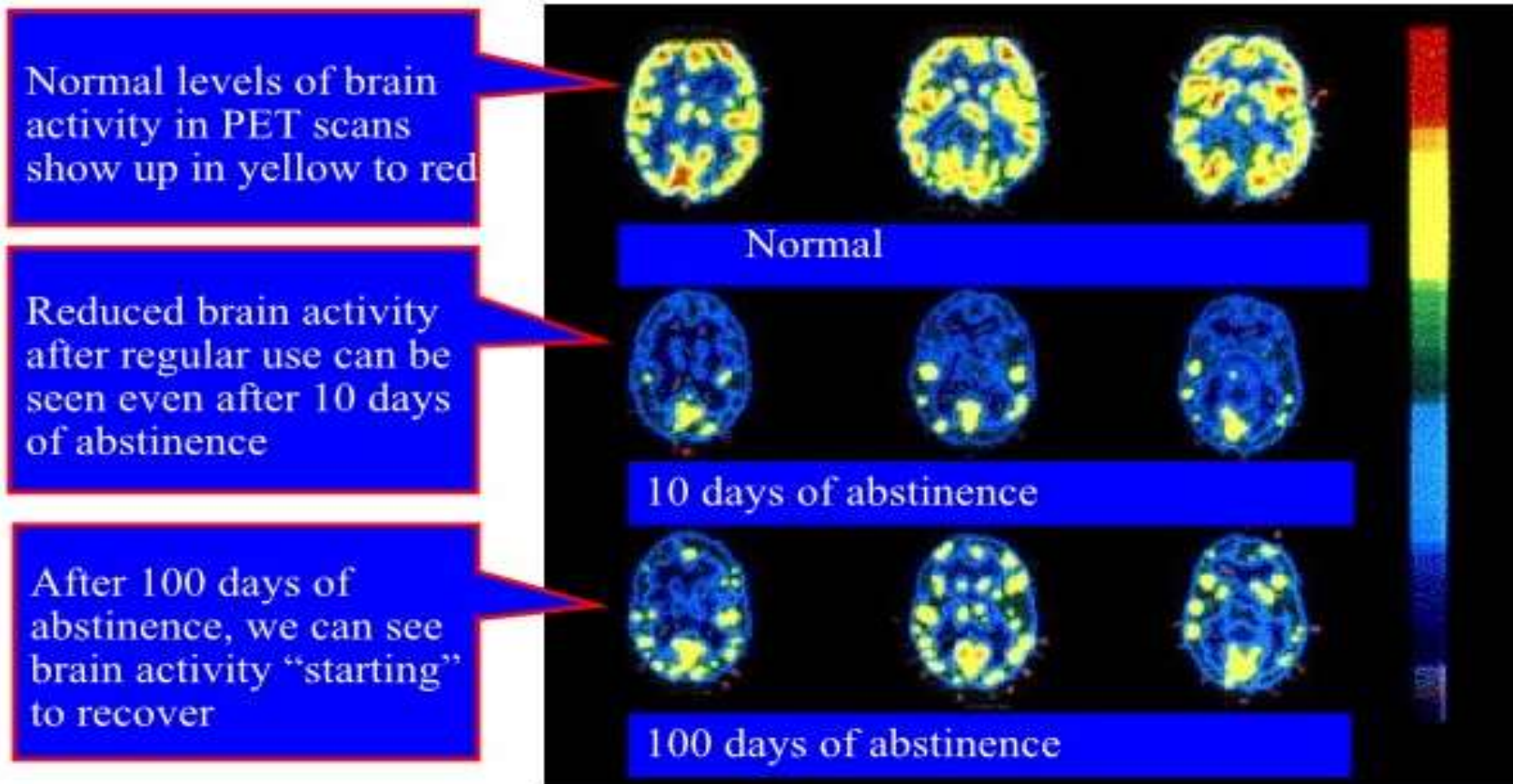


Source: NIDA

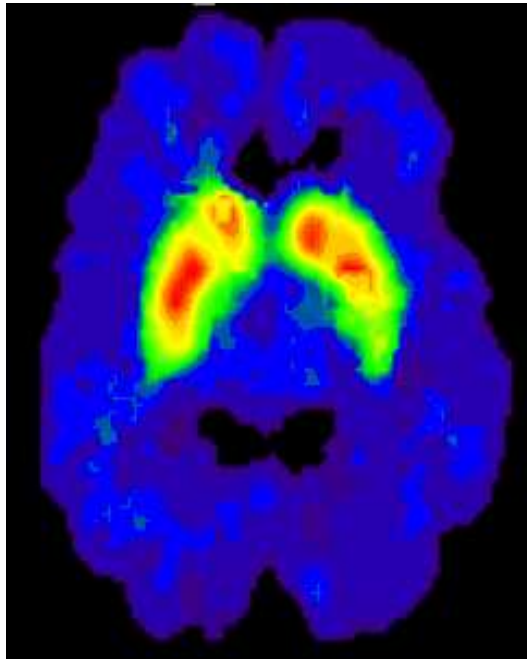
A Brain on Drugs



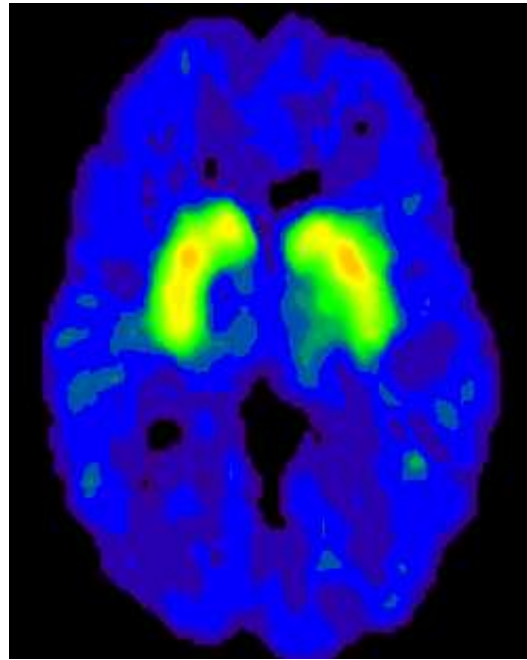
Brain Healing Takes Time



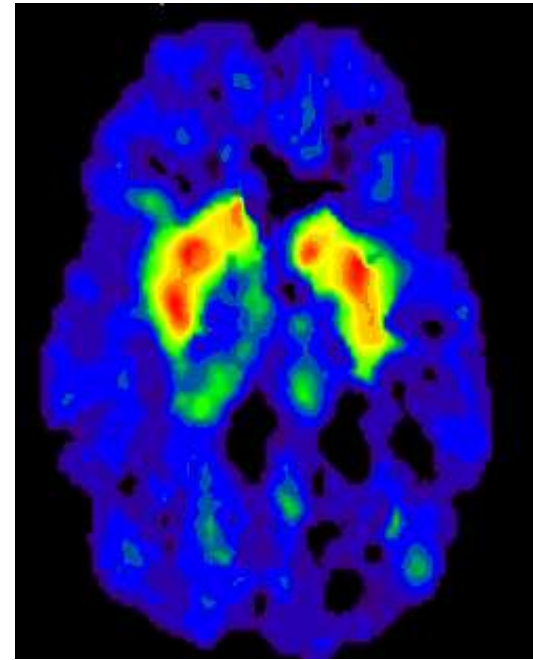
Partial Recovery of Brain Dopamine Transporters in Methamphetamine (METH) Abuser after Protracted Abstinence



Normal Control

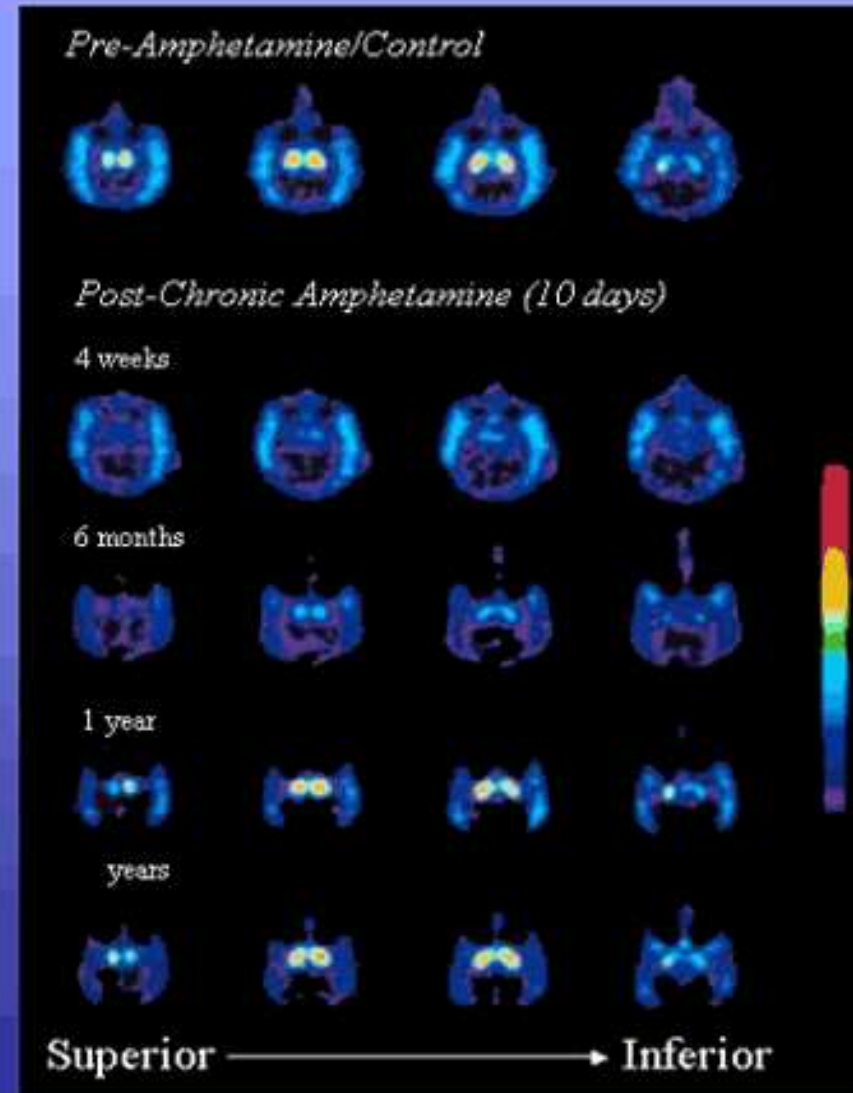


METH Abuser
(1-month detox)



METH Abuser
(24-months detox)

Drugs Have Long-term Consequences



CLINICAL VIGNETTE

Friday at 2:00pm Dr. Ray, a radiologist asks Dr. Sap to write a prescription for his pain. Dr. Ray has low back pain and is driving to Florida in the morning for a family vacation. He can't get to his own doctor because of the interventional ad-ons, one of which is your patient. You write a prescription for hydrocodone/APAP 7.5/325mg, sig: I to II TID, #18 thinking this won't hurt anyone and it less than the 3 day, 180 MME rule.

CLINICAL VIGNETTE

- What are the red flags?
 1. Did not query the CSMD.
 2. Did not get a history.
 3. Did not do a physical examination.
 4. Did not start with a non-opioid analgesic, physical therapy, or other adjunctive care.
 5. Did not document, does not have a patient chart for Dr. Ray

CLINICAL VIGNETTE

In a heated argument over Dr. Ray's drug use, his wife calls the medical board. Apparently, Dr. Ray has been using hydrocodone for many months in increasing amounts and has become noticeably impaired. During the BME investigation they ask Dr. Sap for Dr. Ray's chart. Dr. Sap spends \$23,000 on legal fees only to have his medical license reprimanded which is reported to the NPDB.

Dr. Ray was referred to the Physician Health Program

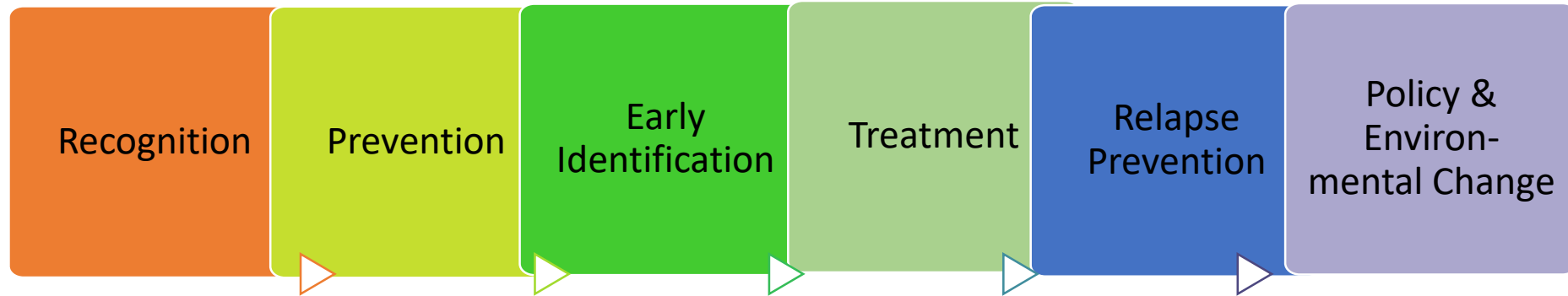
Substances of Addiction

- Alcohol
- Opiates
- Benzodiazepines and Hypnosedatives
- Cocaine, Amphetamines
- Cannabinoids
- Hallucinogens
- Inhalants
- Tobacco
- Etc.

Process Addictions

- Gambling
- Sex, Love, Relationship
- Work
- Food
- Internet
- Video Game
- Gaming Disorder (W.H.O. added for ICD 11)
- Anything that changes how one feels

Approaches to SUDs



Screening & Assessments for Substance Use

Screening

Determining the possibility
that a condition exists

Assessment

Confirming the existence of
a condition and its severity

- History of Present Illness:
- Trauma History:
- Family History:
- Substance History:
- Social History:
- Legal History:
- Recovery History:
- Toxicology:

Goals of Screening

- Both hazardous/harmful drinking or drug use.
- At risk patients for aberrant behavior.
- Patients likely to become substance users.
- Patient substance use, misuse, and problematic use

Types of Screening Tools

Pre-Screening and Screening Tools

- Self-report
- Interview

Self- administered questionnaires

Biological markers – Only recent use

- Breathalyzer + Blood alcohol levels
- Saliva, urine, hair, nail testing
- Serum drug testing

Screening Instruments

Screening Tool	Alcohol	Drug	Adult	Adolescent
NIDA Drug Use Screening tool	✓	✓	✓	✓ with modifiers
CRAFFT	✓	✓		✓
AUDIT	✓		✓	
Opioid Risk Tool		Opioid	✓	✓
CAGE-AID	✓	✓	✓	✓

CAGE-AID

	YES	NO
1. Have you ever felt that you ought to cut down on your drinking or drug use?	☐	☐
2. Have people annoyed you by criticizing your drinking or drug use?	☐	☐
3. Have you ever felt bad or guilty about your drinking or drug use?	☐	☐
4. Have you ever had a drink or used drugs first thing in the morning to steady your nerves or to get rid of a hangover?	☐	☐

A YES answer indicates a possible Substance Use Disorder and the need for further testing.

One Yes = SUD
Sensitivity - 0.79
Specificity - 0.77

Opioid Risk Tool (ORT)

Mark each box that applies		Female	Male
1. Family history of substance abuse	☐ Alcohol ☐ Illegal drugs ☐ Prescription drugs	☐ 1 ☐ 2 ☐ 4	☐ 3 ☐ 3 ☐ 4
2. Personal history of substance abuse	☐ Alcohol ☐ Illegal drugs ☐ Prescription drugs	☐ 3 ☐ 4 ☐ 5	☐ 3 ☐ 4 ☐ 5
3. Age (mark box if 16-45 years)		☐ 1	☐ 1
4. History of preadolescent sexual abuse		☐ 3	☐ 0
5. Psychological disease	☐ Attention-deficit/hyperactivity disorder, obsessive-compulsive disorder, bipolar disorder, schizophrenia ☐ Depression	☐ 2 ☐ 1	☐ 2 ☐ 1
Low (0-3) Moderate (4-7) High (≥8)	Scoring totals	☐	☐

Scoring patients:

- low risk (0-3)
- medium (4-7)
- high (≥ 8)

High risk:
91% sensitivity
for Aberrant
Drug Related
Behavior

Opioid Risk Tool (ORT)

- Self-report screening tool.
- Administered to patients upon an initial visit prior to beginning opioid therapy.
- Can be administered and scored $<$ than 1 min.
- Validated in male and female patients.
- ≤ 3 low risk for future opioid abuse.
- 4 to 7 moderate risk for opioid abuse.
- ≥ 8 a high risk for opioid abuse

Collaborative Requirements for Physicians – APRNs & PAs

- Collaborative Agreement
 - For PAs - Changes filed within 15 days
 - For APRNs - Changes are to be filed within 30 days
- Protocols on Site, signed by both parties
- 20% of charts signed every 30 days.
- 100% of charts reviewed in 10 days and signed within 30 days when a controlled substance is prescribed
- Site visit every 30 days – may change
- Qualified in same specialty
- 2-hour CME to include Chronic Pain Guidelines



CLINICAL



VIGNETTE



Mrs. V. comes to your walk-in, urgent-care clinic reporting that she ran out of her pain pills yesterday. She reports that she was Rx'ed oxycodone ER 20mg TID, #90, 2 weeks ago. She underwent an L4-L5 laminectomy 18 months ago and is wearing a back brace. She is traveling to Florida in the morning. Her UDS is positive for hydromorphone. She reports she ran out because she needed higher doses to control her pain. She is frail, diaphoretic, c/o diarrhea and is easily agitated.

CLINICAL VIGNETTE

- What are the red flags?
 1. She ran out of medications early.
 2. Pseudo-addiction: not a DSM disorder.
 3. UDS: Need to know positives and negatives.
 4. Recognition of early withdrawal

Checking the CSMD

Check the CSMD:

- ☐ Before prescribing an opioid or benzodiazepine as a new course of treatment lasting more than 3 days
- ☐ At least annually when a controlled substance remains part of the patient's treatment plan
- ☐ If doctor shopping, diversion or other misuse is suspected
- ☐ Prescribers should regularly obtain their own practitioner report for unauthorized use or for incorrect information

Not Required to Check the CSMD:

- ☐ The controlled substance is prescribed or dispensed for a patient who is currently receiving Hospice Care
- ☐ Administered directly to a patient during the course of inpatient or residential treatment in a licensed hospital or nursing home

CSMD Requirements

The CSMD report can be placed in the medical record

Report may include data from states with criminal penalties for disclosure so be careful!

Document access to the CSMD in the chart including who and when and any action taken as a result of the findings, or no action taken

Clinical Risk Indicators on CSMD

 = 4 Practitioners in last 90 days

 $\geq$ 5 Practitioners in last 90 days

 = 4 Pharmacies in last 90 days

 $\geq$ 5 Pharmacies in last 90 days

 $\geq$ 90 but < 120 Active MME's

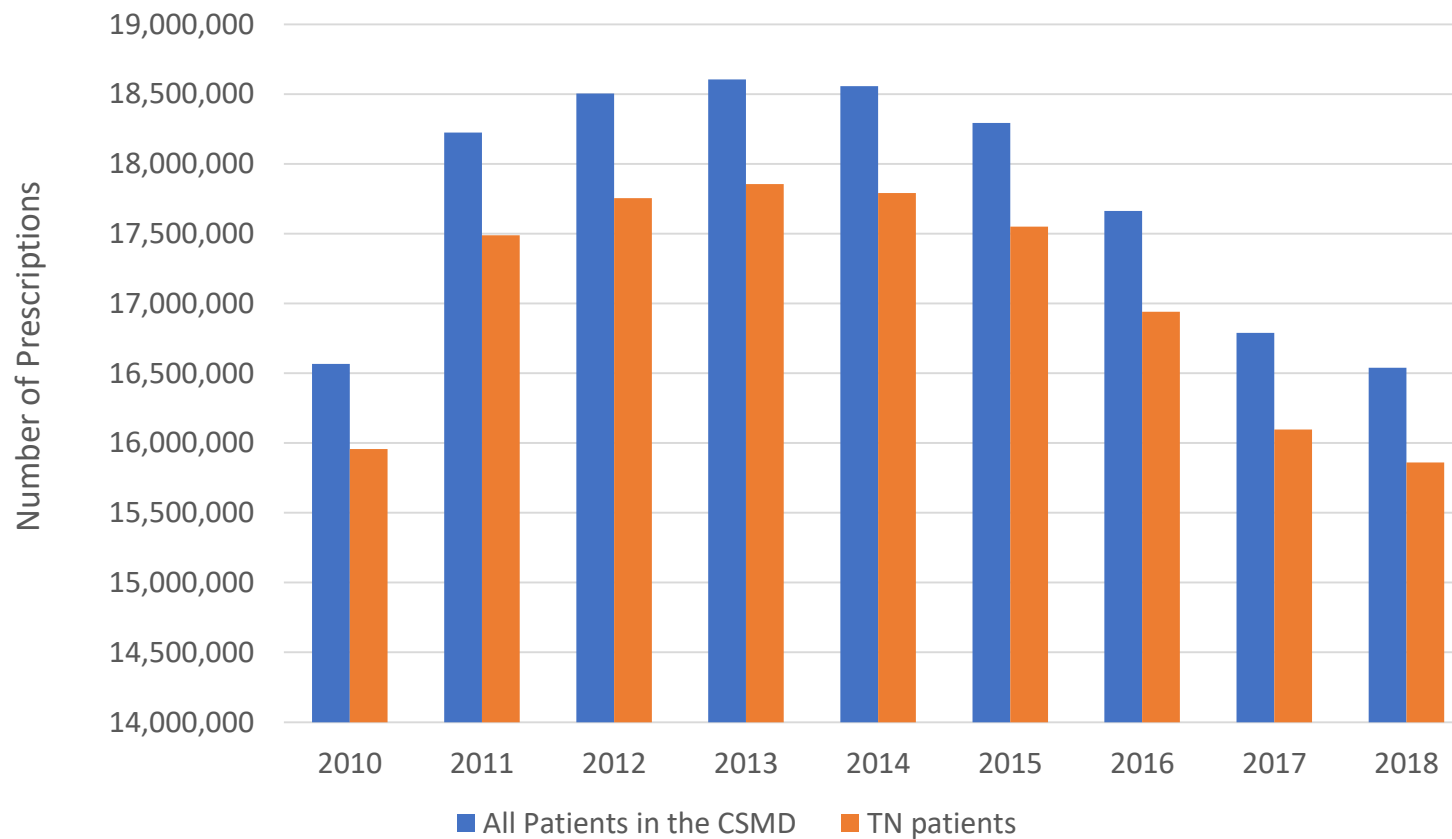
 $\geq$ 120 Active Cumulative MME's

MME = Morphine
Milligram Equivalents

 = High-Risk Female Patient
NAS or NOWS Risk
Female and child-bearing age 15-45

Number of Prescriptions Dispensed in TN & Reported to CSMD

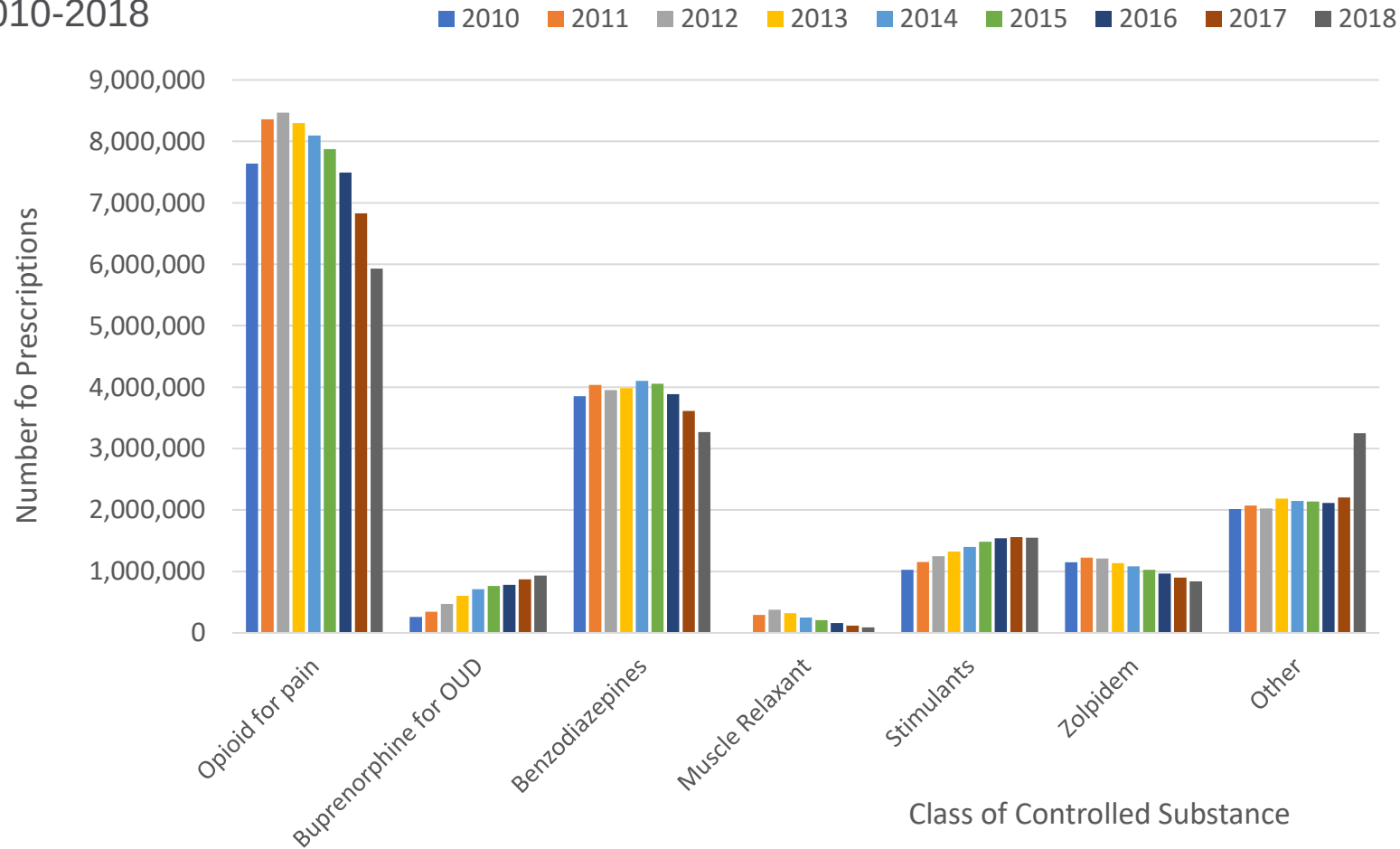
2010-2018



*Excluding prescriptions reported from VA pharmacies.

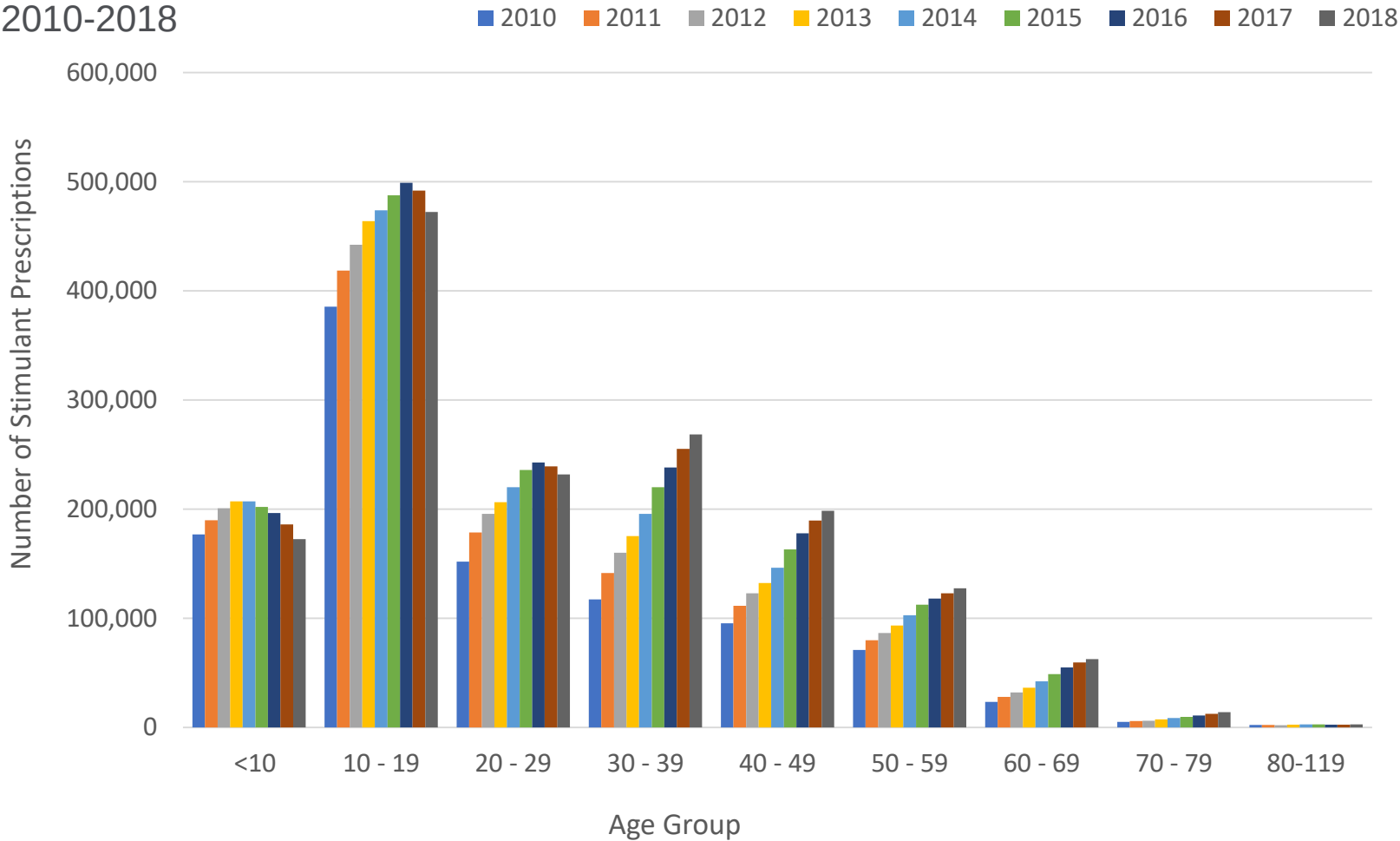
Number of Prescriptions Dispensed by Class & Reported to CSMD

2010-2018



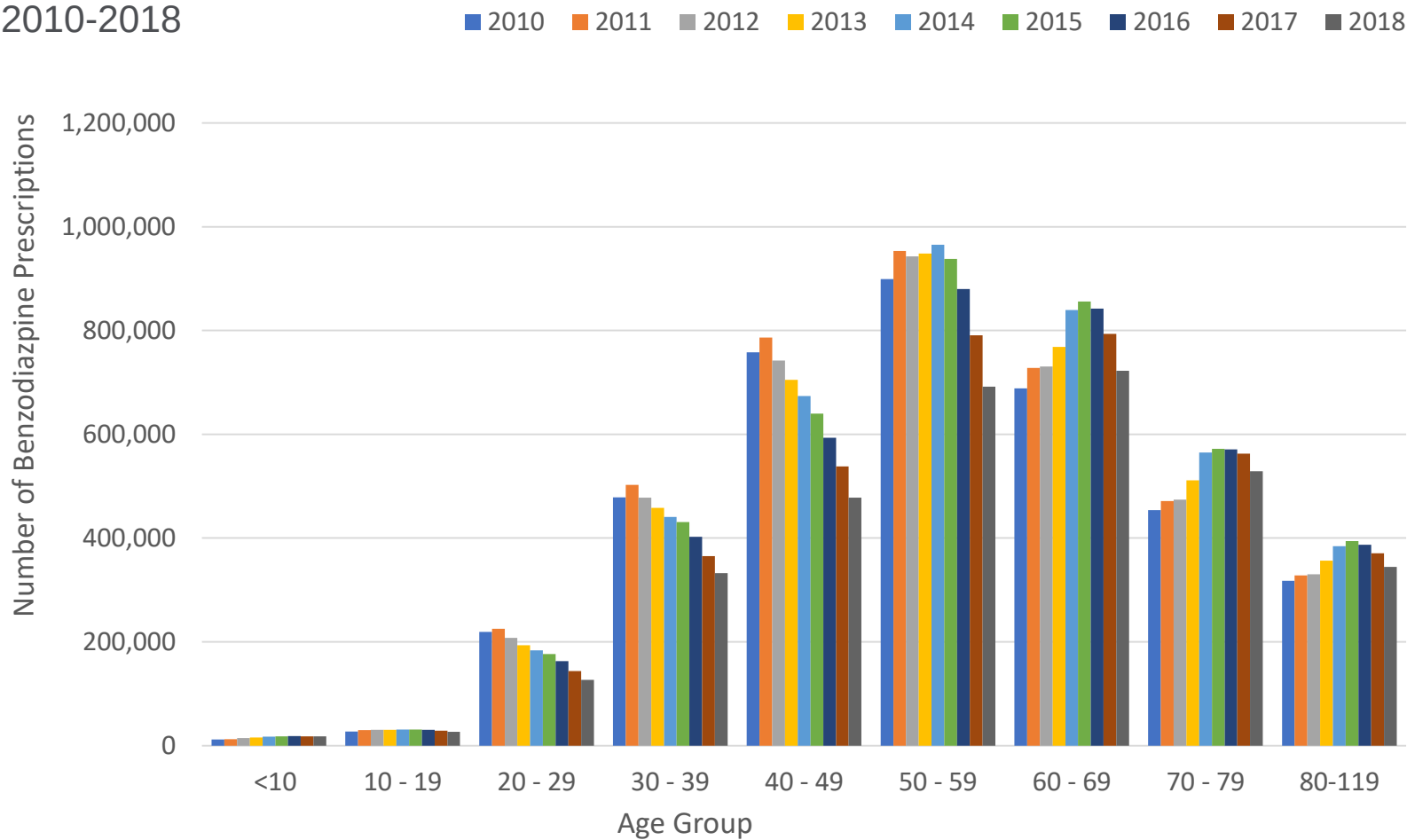
- * 1) The class of controlled substances was defined based on a CDC document. If a drug was not on the document, the drug was grouped into the 'Other';
- 2) Excluding prescriptions reported from VA pharmacies.

Stimulant Prescriptions Dispensed by Age Group & Reported to CSMD



* Excluding stimulant prescriptions reported from VA pharmacies.

Benzodiazepine Prescriptions Dispensed by Age Group & Reported to CSMD



* Excluding prescriptions reported from VA pharmacies.

MME for Long-Acting Opioids Reported to the CSMD

Amount of MME for Long Acting Drugs Dispensed in TN and Reported to the CSMD, 2010-2018*			
Year	Overall patients in CSMD	TN patients	Change among TN patients (%)
2010	3,190,989,273	3,057,991,206	-
2011	3,254,786,743	3,121,293,556	+2.1
2012	3,285,062,156	3,148,353,468	+0.9
2013	3,238,216,544	3,106,161,557	-1.3
2014	2,924,795,127	2,806,107,045	-9.7
2015	2,552,291,111	2,454,148,868	-12.5
2016	2,124,916,097	2,045,899,859	-16.6
2017	1,630,298,000	1,568,894,509	-23.3
2018	1,204,793,575	1,162,067,475	-25.9

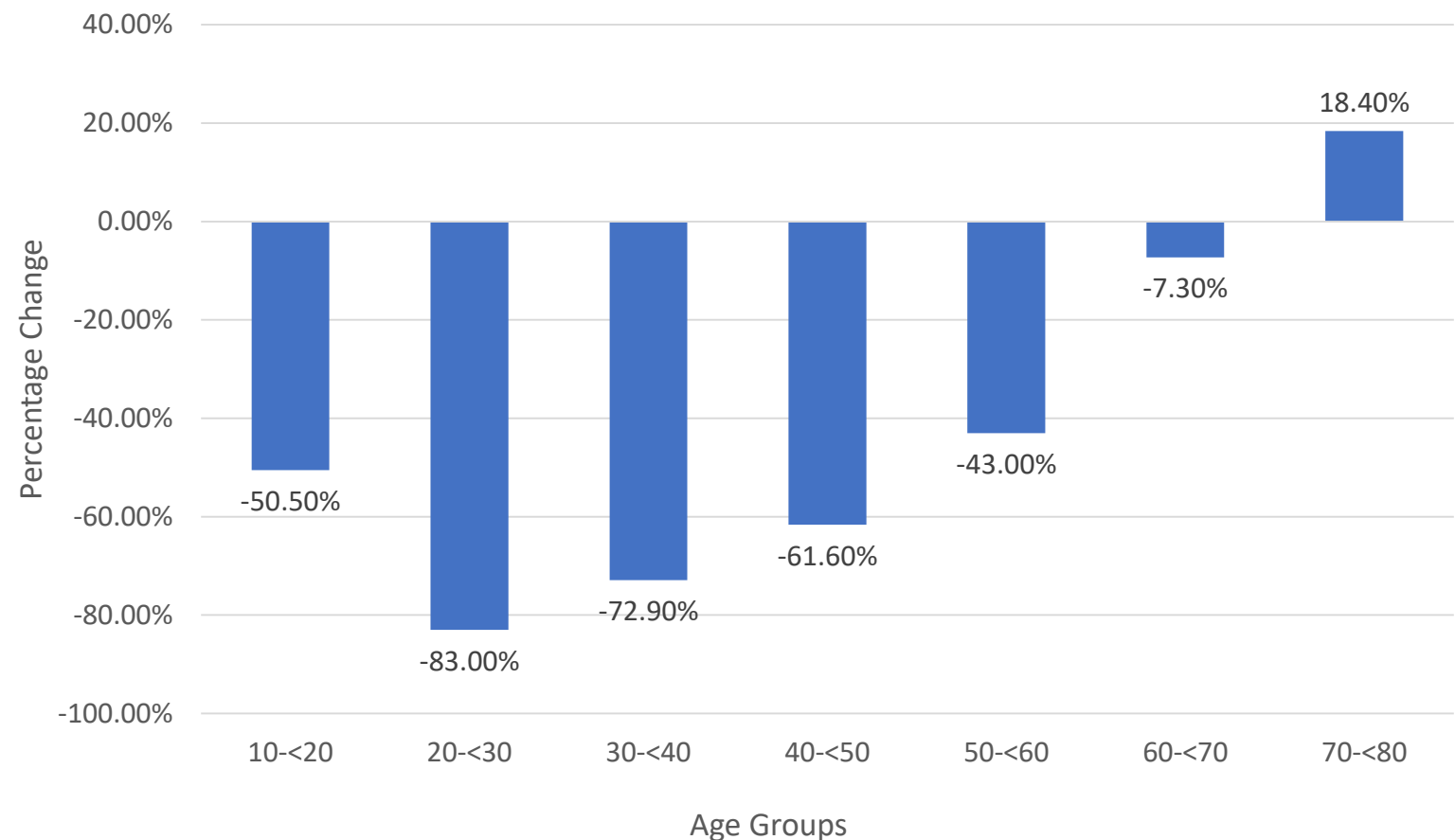
- * 1) The classes of controlled substances were defined based on a CDC document;
 2) Excluding prescriptions reported from VA pharmacies.
 3) Excluding buprenorphine products.

MME for Short-Acting Opioids Reported to CSMD

Amount of MME for Short Acting Drugs Dispensed in TN and Reported to the CSMD, 2010-2018*			
Year	Overall patients in CSMD	TN Patients	Change among TN Patients (%)
2010	5,039,428,269	4,864,582,321	-
2011	5,727,903,926	5,469,306,918	+12.4
2012	5,891,039,406	5,645,050,796	+3.2
2013	5,676,117,306	5,459,300,461	-3.3
2014	5,495,823,563	5,283,695,020	-3.2
2015	5,371,326,766	5,168,525,477	-2.2
2016	5,046,357,775	4,863,320,231	-5.9
2017	4,606,843,191	4,448,492,750	-8.5
2018	4,025,049,294	3,890,868,224	-12.5

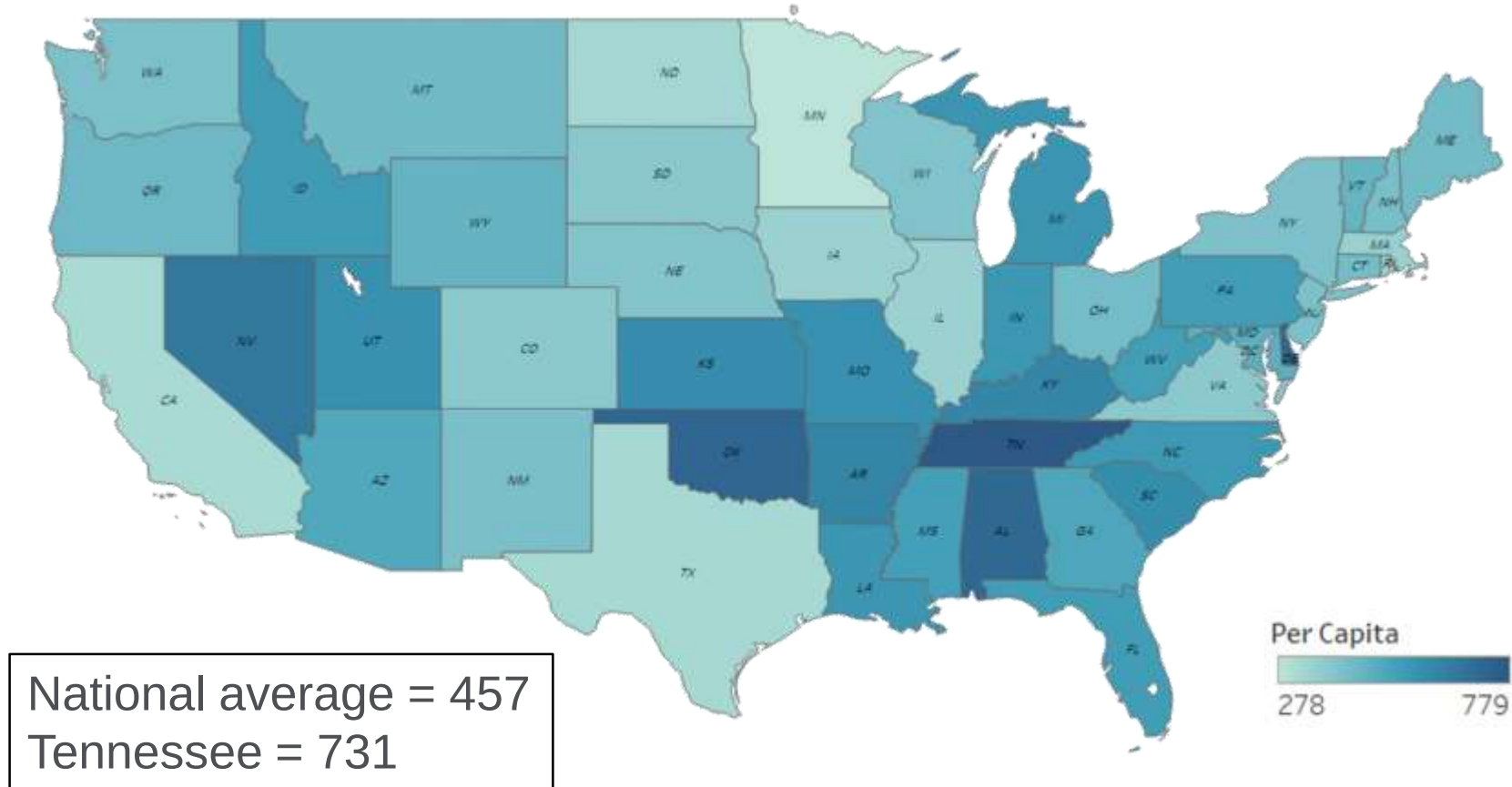
* 1) The classes of controlled substances were defined based on a CDC document;
 2) Excluding prescriptions reported from VA pharmacies.
 3) Excluding buprenorphine products.

Change in MME by Age Group Dispensed & Reported to CSMD



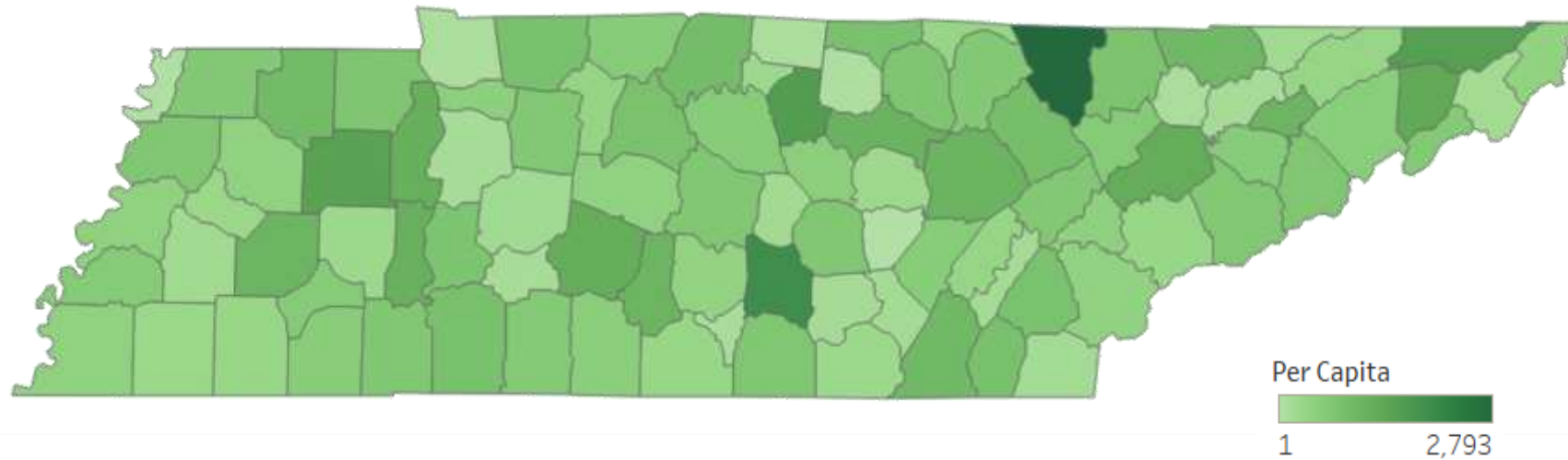
* Excluding prescriptions reported from VA pharmacies; excluding Buprenorphine products.

Opioid Morphine Milligram Equivalents per Capita, 2018



Morphine Milligram Equivalents per Capita Tennessee by County, 2018

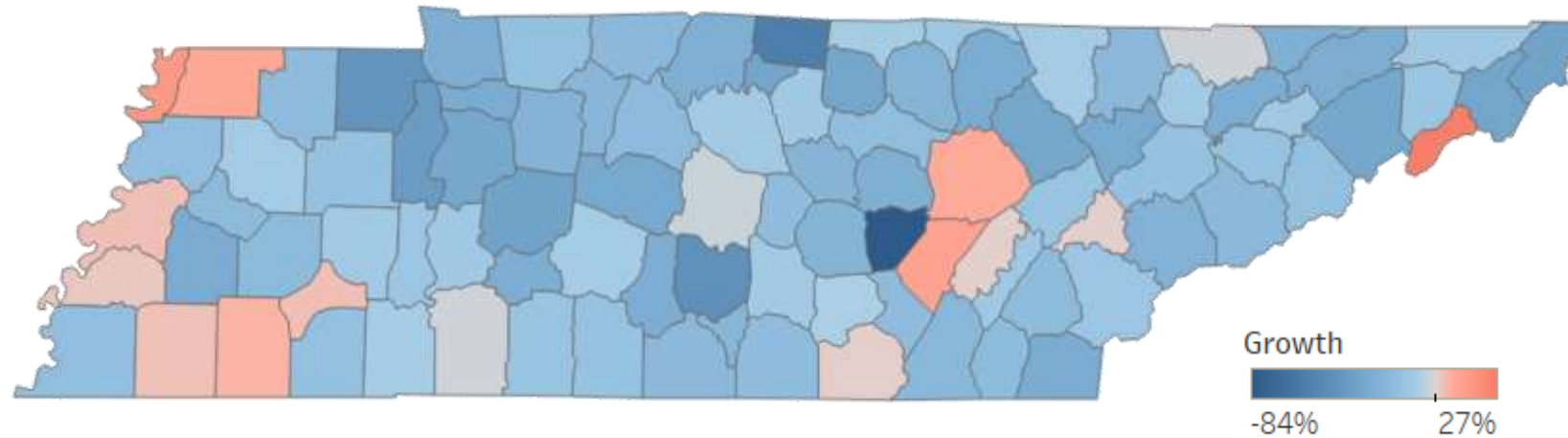
Tennessee MME per capita in 2018 = 731



Change in Morphine Milligram Equivalents per Capita Tennessee by County, 2017-18

Tennessee MME per capita in 2018 = 731
Tennessee MME per capita in 2017 = 887

Rate of change = -16.8%





CLINICAL



VIGNETTE



Ms. S. Eeker is a 26-year-old female who comes in as a new patient. She just moved to town from St Louis. She brings a list of medications and diagnosis:

- hydrocodone/APAP 10/325mg #120, QID PRN.
- alprazolam 2mg, #90, TID PRN.
- zolpidem 10mg, #45, 1-2 QHS.
- carisoprodol, #120, 1 PRN QID.
- duloxetine 60mg, #30, 1 Q Day.

Dx's - Fibromyalgia and Insomnia. As a conflict avoidant, codependent doc you refill her Rx's

CLINICAL VIGNETTE

- What are the red flags?
 1. There is no CSMD report as she is from a suburb of St Louis, Missouri
 2. Old chart?
 3. UDS?
 4. Check with Pharmacy?
 5. Not required to continue any medications
 6. Risk of withdrawal?

Tennessee Regulatory Update



Public Chapter 112

“Nonresidential Buprenorphine
Treatment Guidelines”

DMH & SAS Commissioner develop
guidelines to assist practitioners

Applies to practitioners Rx'ing
buprenorphine-containing products
for Tx of OUD

Public Chapter 978

- Requires prescribers to be licensed as a Non-Residential Office Based Opioid Treatment (OBOT) Program if treat >25% of total patients or >150 patients with buprenorphine for OUD.
 - Effective July 1, 2018

Morphine Milligram Equivalents

Opioid conversion factor to Morphine	Dosage	Frequency	MME/Day	Total MME from a 3-day Rx
Oxycodone 1.5 = 1	5 mg	TID	22.5 MME	67.5 MME
	10mg	TID	45 MME	135 MME
Hydrocodone 1 = 1	5 mg	TID	15 MME	45 MME
	10 mg	QID	40 MME	120 MME
Hydromorphone 4 = 1	4 mg	QID	64 MME	192 MME

What about Tramadol?

The U.S. Food and Drug Administration (FDA) and the National Institutes of Health (NIH) classify Tramadol as an opioid analgesic used for the therapy of mild-to-moderate pain. It is now considered a controlled substance for purposes of reporting to the CSMD.

Tramadol	100mg = 10mg of Morphine (10MME)
----------	----------------------------------

PC 1039 and PC 124 Requirements

4 categories for prescribing opioids:

- I. Up to 3 days or up to 180 MME.
- II. Up to 10 days or up to 500 MME.
- III. Up to 30 days or up to 1200 MME – more than minimally invasive surgery.
- IV. Up to 30 days or up to 1200 MME – Medical Necessity

Exemptions

- Treated with an opioid ≥ 90 days in the last year.
- Active Cancer, Palliative Care, Hospice Care
- Sickle Cell Disease.
- Treated by a pain management specialist.
- Treated for OUD with MAT – Buprenorphine or Methadone.
- Patients with “Severe Burns” or “Major Physical Trauma”.
- Administration in a licensed healthcare facility

Opioid Prescribing Requirements in Tennessee, PC 1039 and PC 124

Requirements for up to a 3 Day or up to a 180 MME Rx

Check the CSMD	Use sound medical judgment
Conduct thorough evaluation of patient	Use sound medical judgment
Obtain informed consent	Use sound medical judgment
Include ICD-10 on chart and Rx	Not required
Document consideration of alternative Tx and why risk of pain exceeds risk of developing a SUD or an OD event	Not required
Additional information on RX	Not required

Opioid Prescribing Requirements in Tennessee, PC 1039 and 124

Requirements for up to 10 Day or up to 500 MME Rx	If Necessary for Acute Pain
Check the CSMD	✓
Conduct thorough evaluation of patient	✓
Obtain written informed consent	✓
Include ICD-10 on chart and Rx	✓
Document consideration of alternative Tx and why risk of pain exceeds risk of developing a SUD or an OD event	Not required
Additional information on RX	Not required

Opioid Prescribing Requirements in Tennessee, PC 1039 and PC 124

Requirements up to 30 Day or up to 1200 MME	More than Minimally Invasive Surgery
Check the CSMD	✓
Conduct thorough evaluation of patient	✓
Obtain written informed consent	✓
Include ICD-10 on chart and Rx	✓
Document consideration of alternative Tx and why risk of pain exceeds risk of developing a SUD or an OD event	✓
Additional information on RX	“Surgery”

Opioid Prescribing Requirements in Tennessee, PC 1039 and PC 124

Requirements up to 30 Day or up to 1200 MME	Medical Necessity Exemption
Check the CSMD	✓
Conduct thorough evaluation of patient	✓
Obtain written informed consent	✓
Include ICD-10 on chart and Rx	✓
Document consideration of alternative Tx and why risk of pain exceeds risk of developing a SUD or an OD event	✓
Additional information on RX	“Medical Necessity”

Informed Consent

Requirements before Rx'ing > 3 day or 180 MME total amount

- Prescriber must sufficiently explain and disclose adequate information to allow the patient to make a knowing and voluntary decision to give written consent for opioid therapy.
 - Must include: Risks, effects, and characteristic of opioids
 - including risks of physical dependency, addiction, misuse and diversion
 - What to expect when taking an opioid and how opioids should be used
 - Reasonable alternatives to opioids
- Discussion of whether the patient should take opioids.
- Women of childbearing age (15-44) and ability.
 - Information regarding neonatal abstinence syndrome (NAS)
 - methods of birth control
 - availability of free or reduced cost birth control
- A reasonable opportunity for questions by the patient

PC 124

- Requires all Schedules II, III, IV & V to be electronically prescribed - effective Jan. 1, 2021.
 - Few exceptions – technological issue, out of state pharmacy, write < 50 Schedule II Rxs, waiver from the DOH, others – see **Tenn. Code Ann. § 63-1-160(d)**
- Defined **Severe Burn, Major Physical Trauma** and **Palliative Care**.
- Lengthens the 3-day limit to 14 days for the treatment of an Upper Respiratory Infection
 - Codeine preparations for cough

PC 124

Palliative Care

Specialized treatment for patients facing serious illness, which focuses on providing relief of suffering through a multidisciplinary approach to maximize quality of life.

Serious Illness

Health condition that carries a high risk of mortality and negatively impacts a patient's daily bodily functions.

Severe Burn

Injury sustained from thermal or chemical causes resulting in second degree or third-degree burns.

Major Physical Trauma

Serious injury sustained due to blunt or penetrating force resulting in serious blood loss, fracture, significant temporary or permanent impairment, or disability

New HHS Tapering of Long-Term Opioid Analgesics

Risks of Rapid Opioid Taper or Sudden Discontinuation:

- Opioids shouldn't be tapered rapidly or discontinued suddenly due to risks of significant opioid withdrawal.
- Risk: acute withdrawal symptoms, pain exacerbation, serious psychological distress (suicidality).
- Patients may seek other sources of opioids (including illicit), to treat their pain or withdrawal symptoms.
- Absent indications of a life-threatening issue, such as warning signs of impending overdose, HHS does not recommend abrupt opioid dose reduction or discontinuation

HHS Tapering of Long-Term Opioid Analgesics

Consider tapering to a reduced opioid dosage, or tapering and discontinuing opioid therapy, when:

- Pain improves.
- The patient requests dosage reduction or discontinuation.
- Pain and function are not meaningfully improved.
- The patient is receiving higher opioid doses without evidence of benefit from the higher dose.
- The patient has current evidence of opioid misuse.
- The patient experiences side effects that diminish quality of life or impair function

HHS Tapering of Long-Term Opioid Analgesics

- The patient experiences an overdose or other serious event (e.g., hospitalization, injury), or has warning signs for an impending event such as confusion, sedation, or slurred speech.
- The patient is receiving medications (e.g., benzodiazepines) or has medical conditions (e.g., lung disease, sleep apnea, liver disease, kidney disease, fall risk, advanced age) that increase risk for adverse outcomes.
- The patient has been treated with opioids for a prolonged period (e.g., years), and current benefit-harm balance is unclear

HHS Tapering of Long-Term Opioid Analgesics

Important considerations prior to deciding to taper:

- Avoid insisting on tapering or discontinuation when opioid use may be warranted (benefits outweigh risks).
- Avoid misinterpreting cautionary dosage thresholds as mandates for dose reduction.
- Some patients using both benzodiazepines and opioids may require tapering one or both medications to reduce risk for respiratory depression.
- Avoid dismissing patients from care.
- There are serious risks to noncollaborative tapering in physically dependent patients including:
 - acute withdrawal, pain, anxiety, depression, suicidality, self-harm & seeking opioids from high-risk sources

HHS Tapering of Long-Term Opioid Analgesics

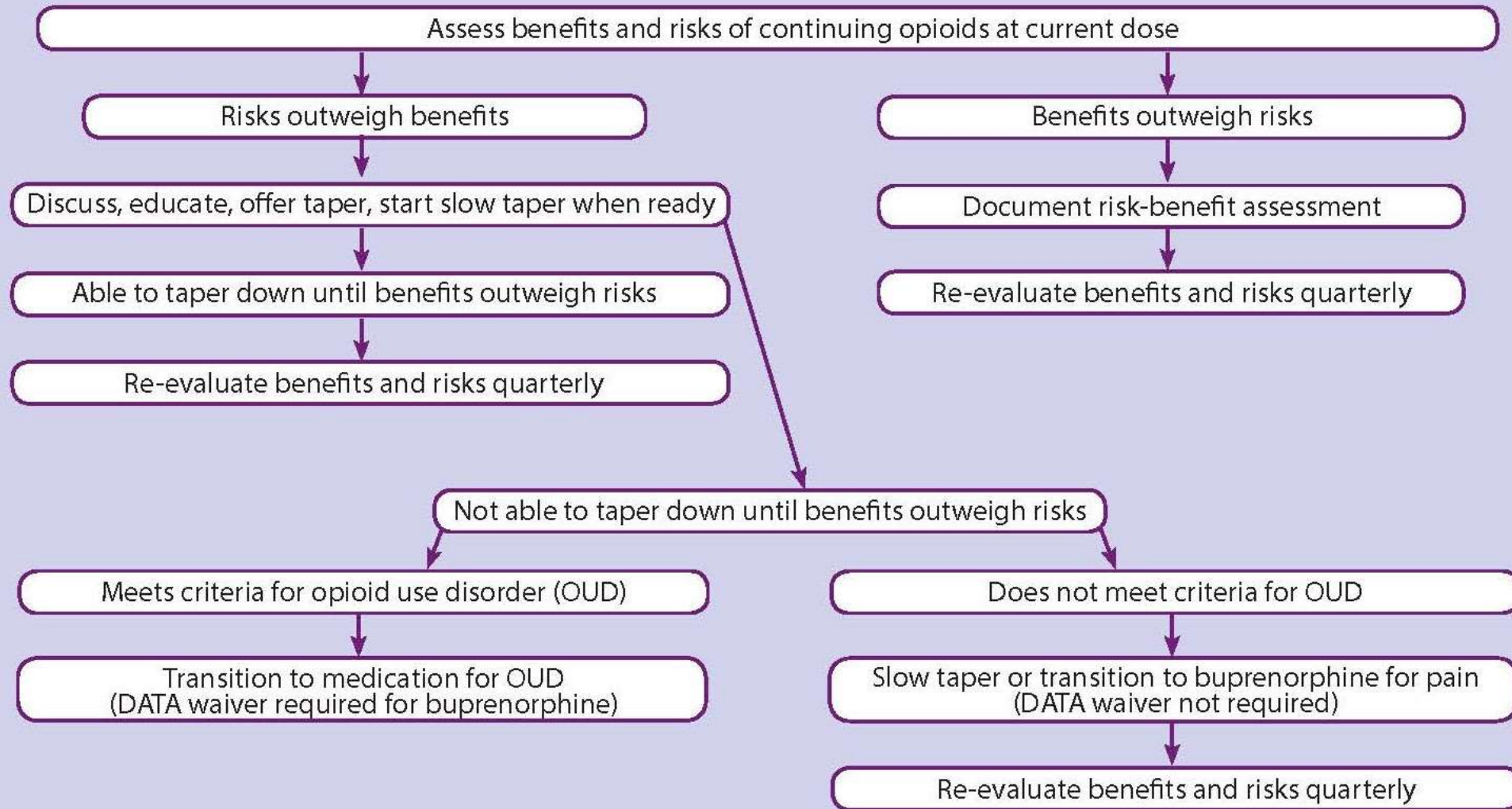
Important steps prior to initiating a taper:

- Commit to working with your patient to improve function and decrease pain.
- Depression, anxiety, and post-traumatic stress disorder (PTSD) can be common in patients with painful conditions, especially in patients receiving long-term opioid therapy. Treat comorbid conditions.
- If your patient has serious mental illness, is at high suicide risk, arrange for consultation with a behavioral health provider before initiating a taper

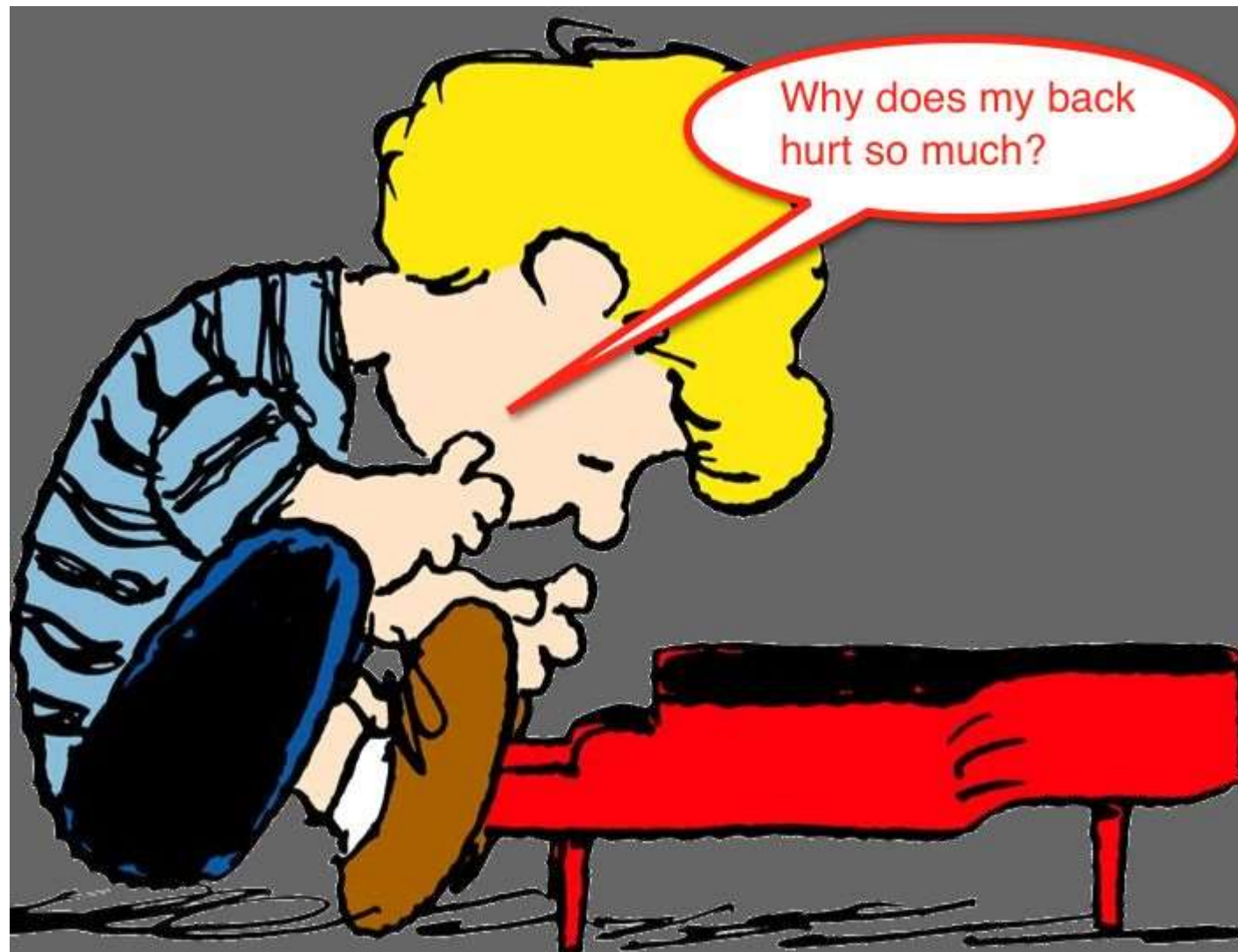
HHS Tapering of Long-Term Opioid Analgesics

- Assess for opioid use disorder including:
 - Opioid misuse behavior or other signs of opioid use disorder. Refer for treatment or arrange for MAT.
- Access appropriate expertise if considering opioid tapering or managing opioid use disorder during pregnancy.
 - Opioid withdrawal risks include spontaneous abortion and premature labor
- Advise patients of increased risk for overdose on abrupt return to a previously prescribed higher dose
 - Strongly caution that tolerance can be lost in a week and risk of overdose if they return to their original dose. Provide overdose education. Consider offering naloxone.

Opioid Tapering Flowchart



Adapted from Oregon Pain Guidance. Tapering – Guidance & Tools. Available at <https://www.oregonpainguidance.org/guideline/tapering/>.



Tennessee Chronic Pain Guidelines



Clinical Practice
Guidelines for Outpatient
Management of Chronic
Non-Malignant Pain
3rd Edition

Tennessee Chronic Pain Guidelines



Indicated for primary care,
not pain specialists



Are accepted medical practice



Available online at Tennessee
Department of Health website

[https://www.tn.gov/content/dam/tn/health/documents/
ChronicPainGuidelines.pdf](https://www.tn.gov/content/dam/tn/health/documents/ChronicPainGuidelines.pdf)

Tennessee Chronic Pain Guidelines

❖ SECTIONS I, II, III

❖ APPENDICIES

- ☐ CORE COMPETENCIES
- ☐ TN TOGETHER
- ☐ PAIN MEDICINE SPECIALIST
- ☐ MENTAL HEALTH ASSESSMENT TOOLS
- ☐ MEDICATION ASSISTED TREATMENT PROGRAM
- ☐ WOMEN'S ISSUES: WOMEN OF CHILDBEARING AGE
- ☐ PREGNANT WOMEN
- ☐ RISK ASSESSMENT TOOLS
- ☐ CSMD:
- ☐ SAMPLE INFORMED CONSENT: Controlled Substance Agreement
- ☐ SAMPLE PATIENT AGREEMENT: Controlled Substance Treatment
- ☐ URINE DRUG TESTING
- ☐ TAPERING PROTOCOL
- ☐ MORPHINE EQUIVALENT DOSE
- ☐ NALOXONE
- ☐ SAFETY NET
- ☐ PRESCRIPTION DRUG DISPOSAL
- ☐ USE OF OPIOIDS IN WORKERS' COMPENSATION MEDICAL CLAIMS
- ☐ MEDICAL TREATMENT GUIDELINES FOR PAIN MANAGEMENT FOR WORKERS' COMPENSATION
- ☐ CHRONIC PAIN GUIDELINE ALGORITHM WOMEN'S HEALTH
- ☐ CHRONIC PAIN GUIDELINE ALGORITHM OPIOID THERAPY
- ☐ NON-OPIOID THERAPIES
- ☐ ACUTE PAIN
- ☐ PERIOPERATIVE PAIN MANAGEMENT
- ☐ TENNESSEE EMERGENCY DEPARTMENT OPIOID PRESCRIBING GUIDELINES
- ☐ PEDIATRIC PAIN
- ☐ TERMS/DEFINITIONS
- ☐ LINKS
- ☐ REFERENCES

Tennessee Chronic Pain Guidelines

Section	
I	Prior to Initiating Opioid Therapy for Chronic Non-Malignant Pain
II	Initiating Opioid Therapy for Chronic Non-Malignant Pain
III	Ongoing Opioid Therapy for Chronic Non-Malignant Pain

Tennessee Chronic Pain Guidelines

I: Prior to Initiating Opioid Therapy:

- Continuation by itself is not a good reason.
- Stepwise approach, using non-opioids first.
- Discuss birth control and pregnancy at each visit.
- Document: H+P, labs and imaging.
- Telemedicine SHALL not be used to treat chronic pain

Tennessee Chronic Pain Guidelines

I: Prior to Initiating Opioid Therapy:

- Evaluation of the pain; nature, intensity, treatments and function.
- Evaluate for comorbidity.
- Document a review of systems.
- Screen for mental health disorders including SUD's.
- Review old medical records

Tennessee Chronic Pain Guidelines

I: Prior to Initiating Opioid Therapy:

- Establish a Dx to justify medication.
- Use assessment tools to determine risk.
- Toxicology screening- urine, hair, nail, etc.
- Check the CSMD.
- Formulate/document a Treatment plan.
- Primary goal is improvement in Function

Tennessee Chronic Pain Guidelines

II: Initiating Opioid Therapy

- Risk of overdose starts at 40 MMED.
- Risk greatly increases @ 80-100 MMED.
- Start low, go slow with IR, use therapeutic trial.
- Avoid benzodiazepines; if BZD are used and >120 MMED refer to mental health.
- Informed consent, treatment agreement.
- Monitor for signs of diversion

Tennessee Chronic Pain Guidelines

III: Ongoing Opioid Therapy

- Use Single provider or practice, single pharmacy.
- Use lowest effective dose.
- Use 1 short acting opioid.
- Document the Five A's.
 - Analgesia.
 - Activities of daily living.
 - Adverse side effects.
 - Aberrant behaviors.
 - Affect

Tennessee Chronic Pain Guidelines

III: Ongoing Opioid Therapy

- > 120 MME DD refer for consultation.
- Check the CSMD.
- Monitor for aberrant behavior.
- Utilize ongoing risk assessments.
- D/C when risks > benefits

Tennessee Chronic Pain Guidelines

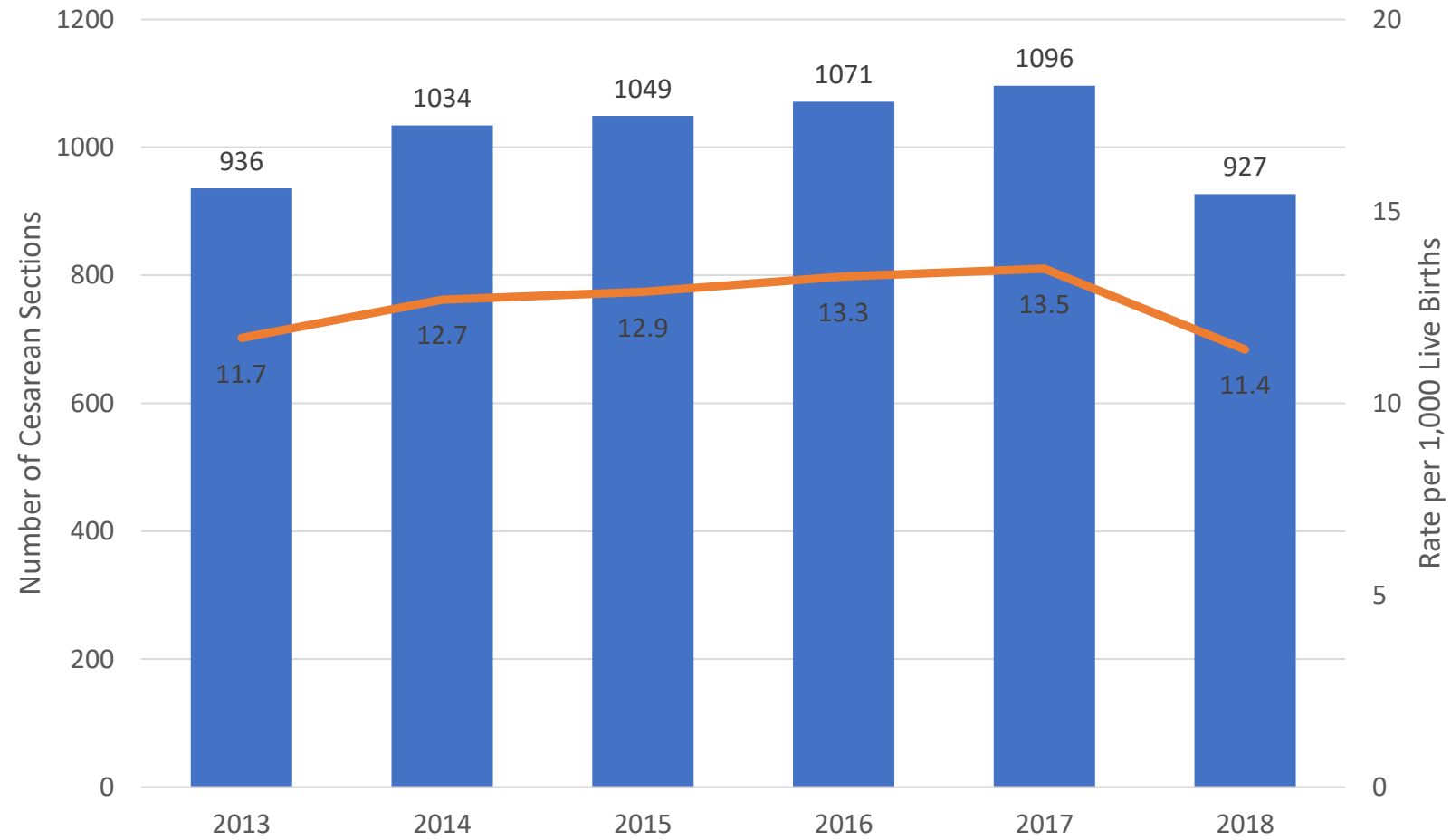
II and III: Initiating and Ongoing Opioid Therapy - Women's Health:

- Discuss birth control at each visit.
- Use consent form about risks of opioids and pregnancy.
- Refer to OB if pregnancy occurs.
- Obtain Urine Pregnancy Test prior to initiating opioids and at follow-up visits

Women's Health - Pregnancy

- A risk assessment, Urine Drug Screen (UDS), and CSMD check should be performed before initiating opioid or benzodiazepine therapy during pregnancy.
- The risks of documented Intra-Uterine Drug Exposure should be discussed and
- A positive aberrant drug history on prenatal visits should prompt a UDS upon admission for delivery to help identify infant at risk for NAS.
- Appropriate discontinuation has been shown to be safe for fetus during pregnancy. However, unintended consequences from tapering may outweigh benefits

Annual NAS Trends



Source: Neonatal Abstinence Surveillance System. TN Dept. of Health .

Financial Costs of NAS

	Non-low Birth Weight Babies	NAS Babies
Average TennCare cost per infant	\$4,944	\$56,965
Average length of stay (days)	2.2	26.8

Data from 2014 TN Health Care Finance Administration..

CLINICAL VIGNETTE

Mr. Pele comes to the E.D. after hurting his foot playing indoor soccer. He has a 5th metatarsal mid shaft spiral fracture confirmed by X-ray. As part of the social history Mr. Pele tells the doctor that he is a recovering alcoholic with 8 years sobriety. He is referred to an orthopedic surgeon and placed in a Bledsoe boot. He is prescribed hydrocodone/APAP, 5mg/325mg, # 21 and told not to worry about the narcotics, “its not booze” Mr. Pele had good pain relief from APAP 1000mg he took prior to coming to ED.

CLINICAL VIGNETTE

- What are the red flags?

Addiction is one disease with many faces. Recovering addicts can be inadvertently triggered by a medication that activates the reward - craving process. By definition all scheduled medications have this potential.

This occurs with way too much frequency.

Mr. Pele throws out the Rx and takes APAP 1000mg with good pain relief

Evidence-Based Medicine

Opioids:

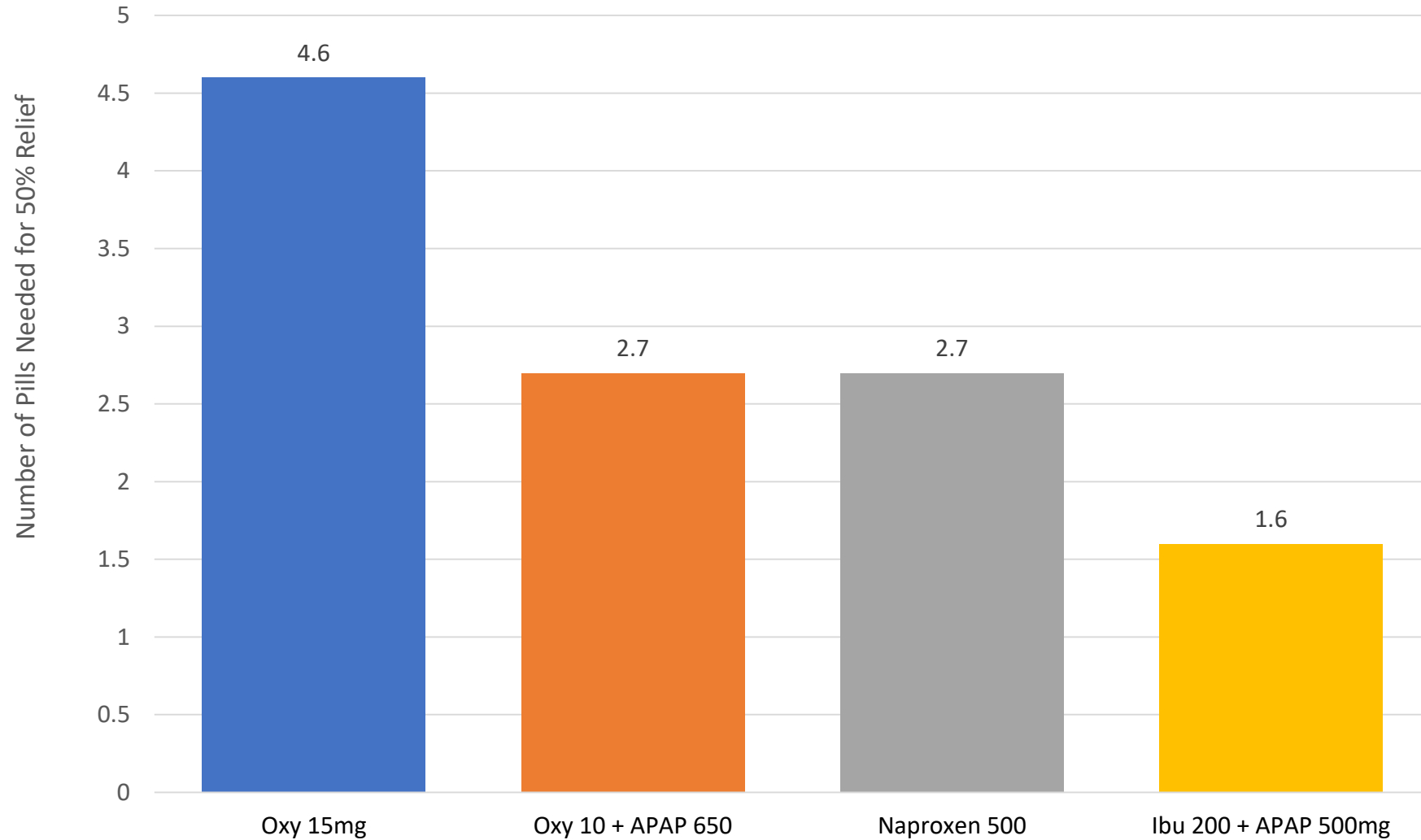
No prospective study has clearly demonstrated long-term safety or long-term efficacy in terms of analgesia or functional improvement.

Evidence-Based Medicine

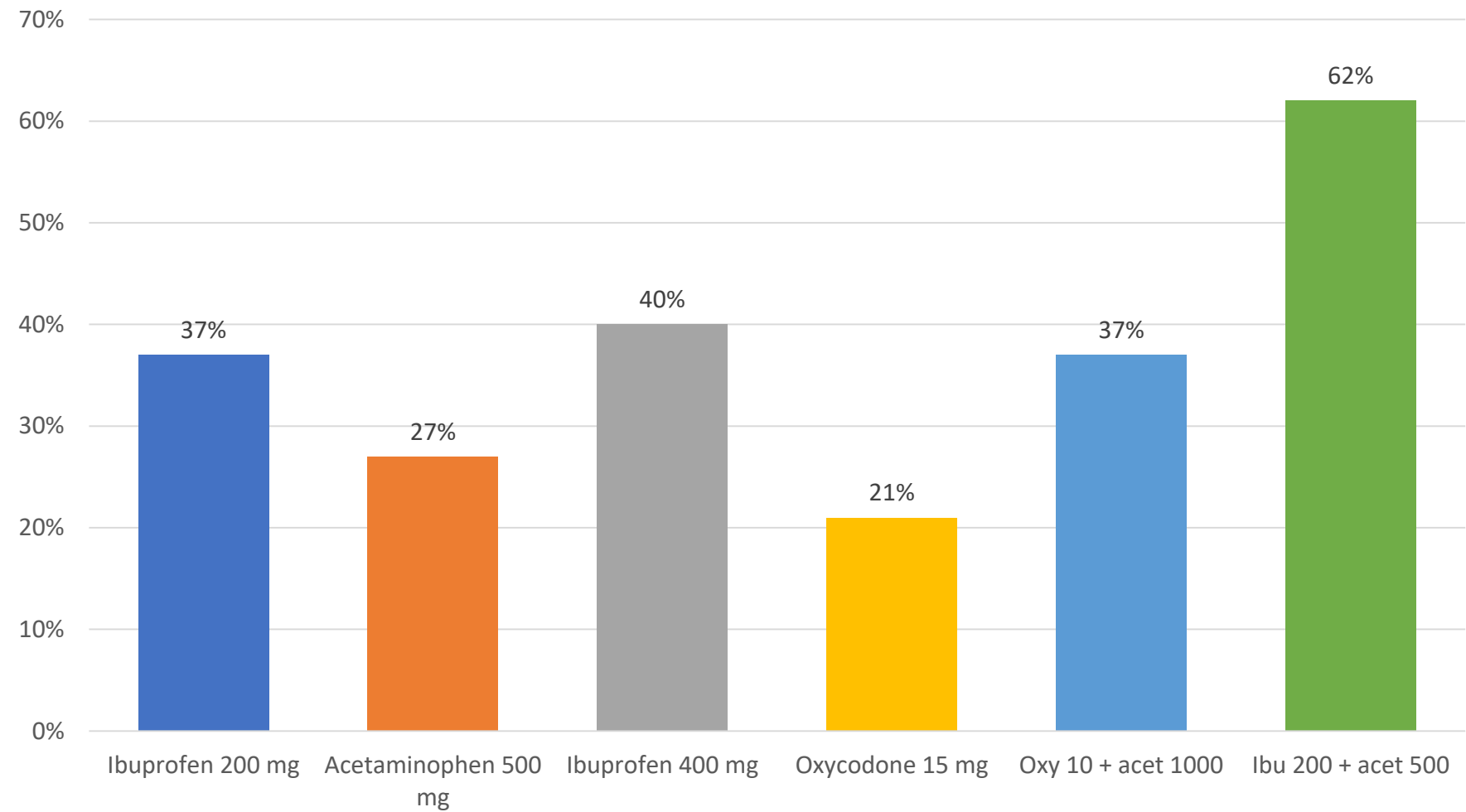
Long-term Opioid Therapy for Neuropathy

“Long-term opioid therapy did not improve functional status but rather was associated with a higher risk of subsequent opioid dependency and overdose.”

Efficacy of Pain Medicine: # Pills for 50% Relief

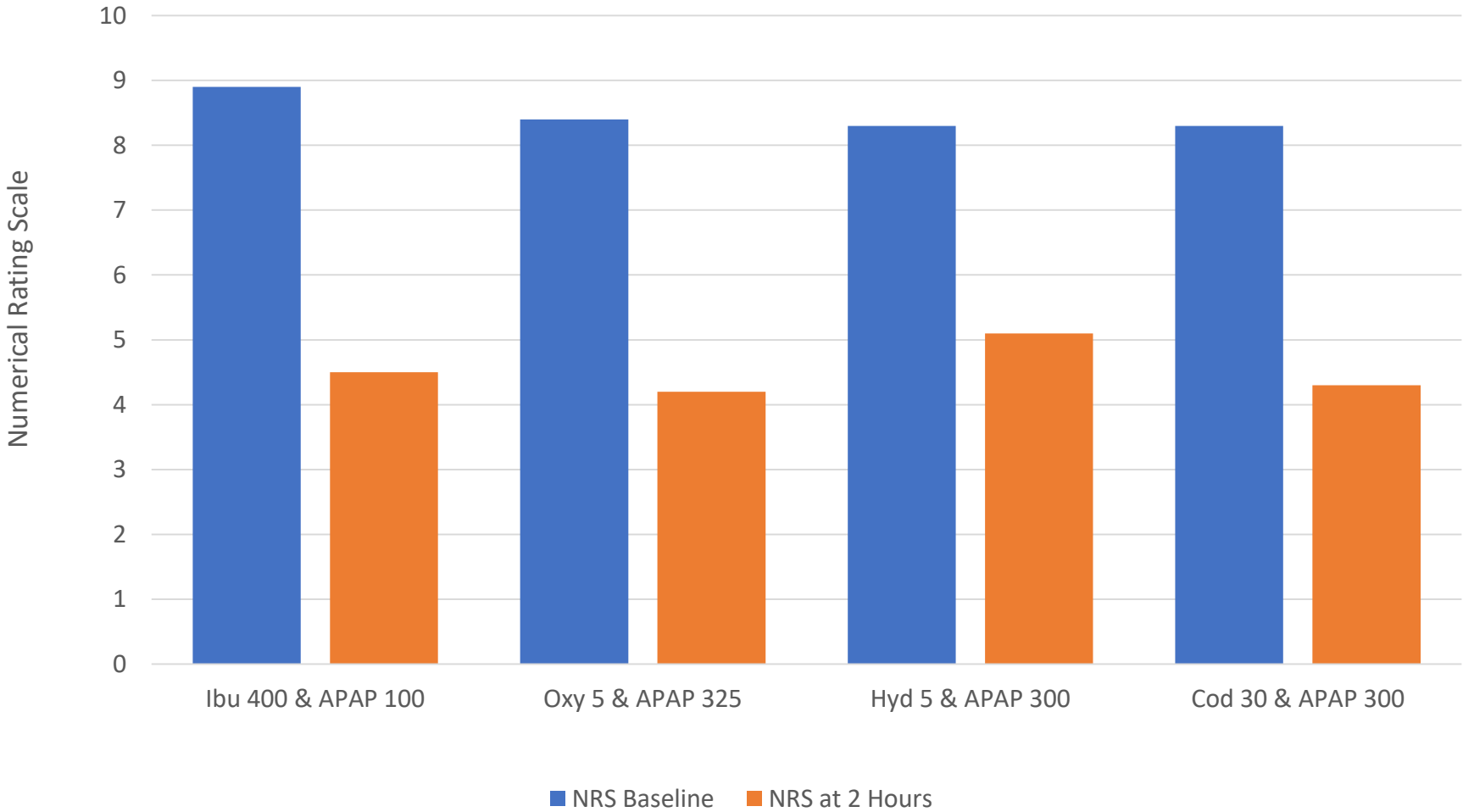


Efficacy of Pain Medicine: Percent with 50% Relief



Bandolier. (2007). The Oxford League Table of Analgesic Efficacy

Efficacy of Pain Medicine: Single Dose Oral Analgesic in ED on Acute Extremity Pain



JAMA. CHANG, 2017;318(17):1661-1667.

Effect of Opioid vs Nonopioid Medications on Pain-Related Function in Patients With Chronic Back Pain or or Knee Osteoarthritis Pain. SPACE Clinical Trial

240 randomized patients – 12 months.

Pain was significantly better in the nonopioid group.

Adverse medication-related symptoms were significantly more common in the opioid group.

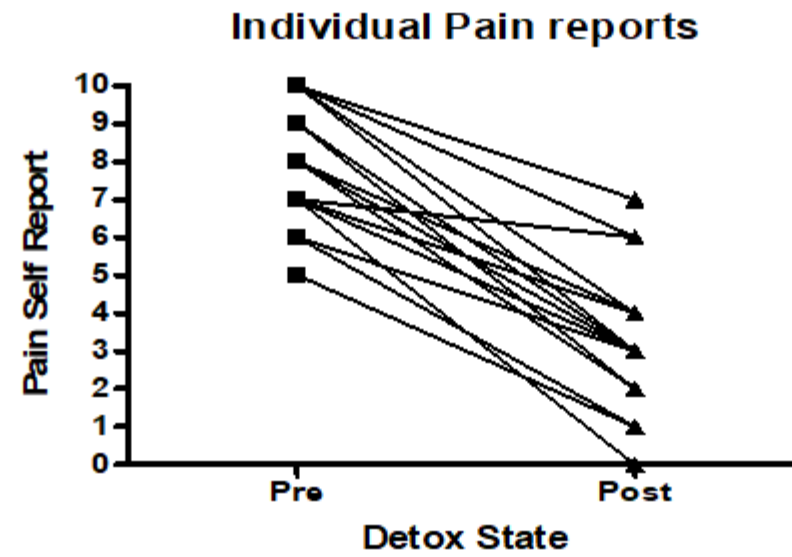
Conclusion: Treatment with opioids was not superior to treatment with nonopioid medications for improving pain-related function over 12 months. Results do not support initiation of opioid therapy for moderate to severe chronic back pain or hip or knee osteoarthritis pain

Original Research

“Significant Pain Reduction in Chronic Pain Patients after Detoxification from High Dose Opiates”

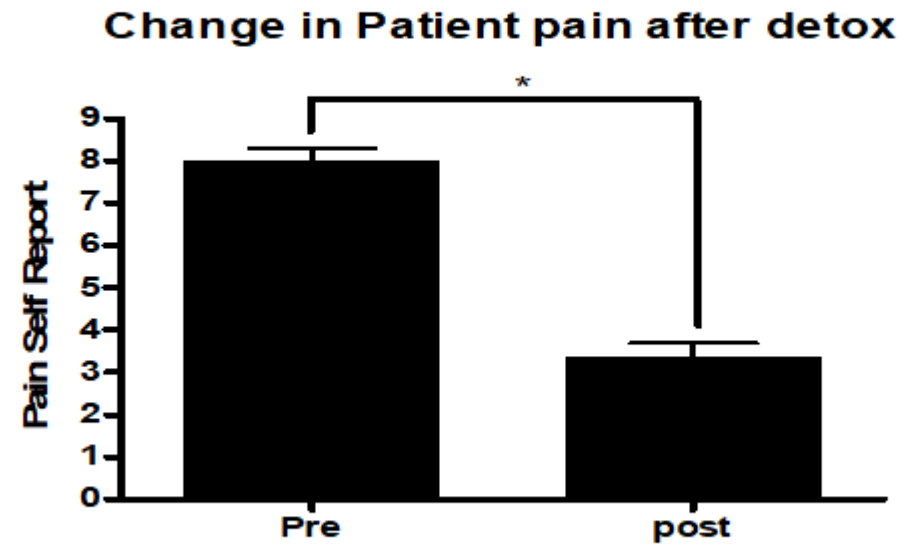
Journal of Opioid Management
2:5 September/October 2006
Michael Baron, MD, MPH

Figure 1.



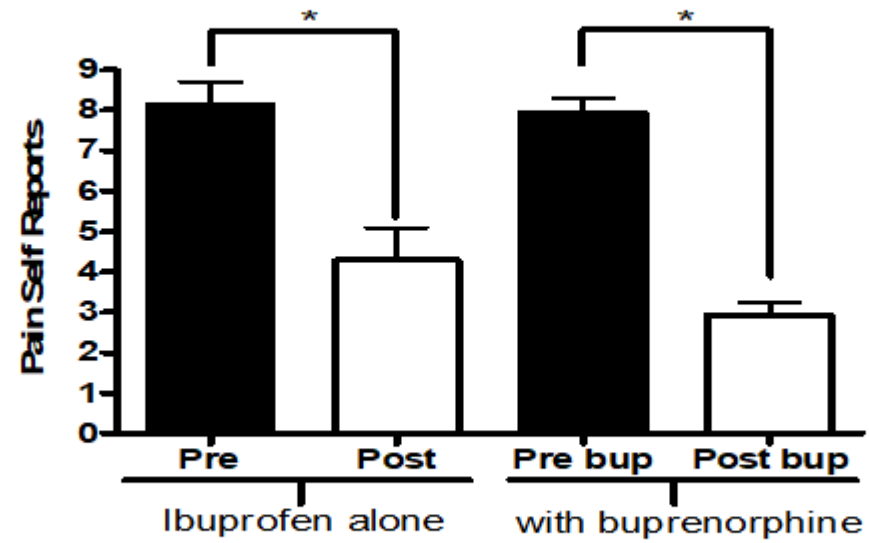
Journal of Opioid Management, September/October, 2006.

Figure 2.



Journal of Opioid Management, September/October, 2006.

Figure 3.



Journal of Opioid Management, September/October, 2006.

Choose treatment options that are appropriate for the patient and for the type of pain.



“Lose some weight, quit smoking, move around more and eat the carrot.”

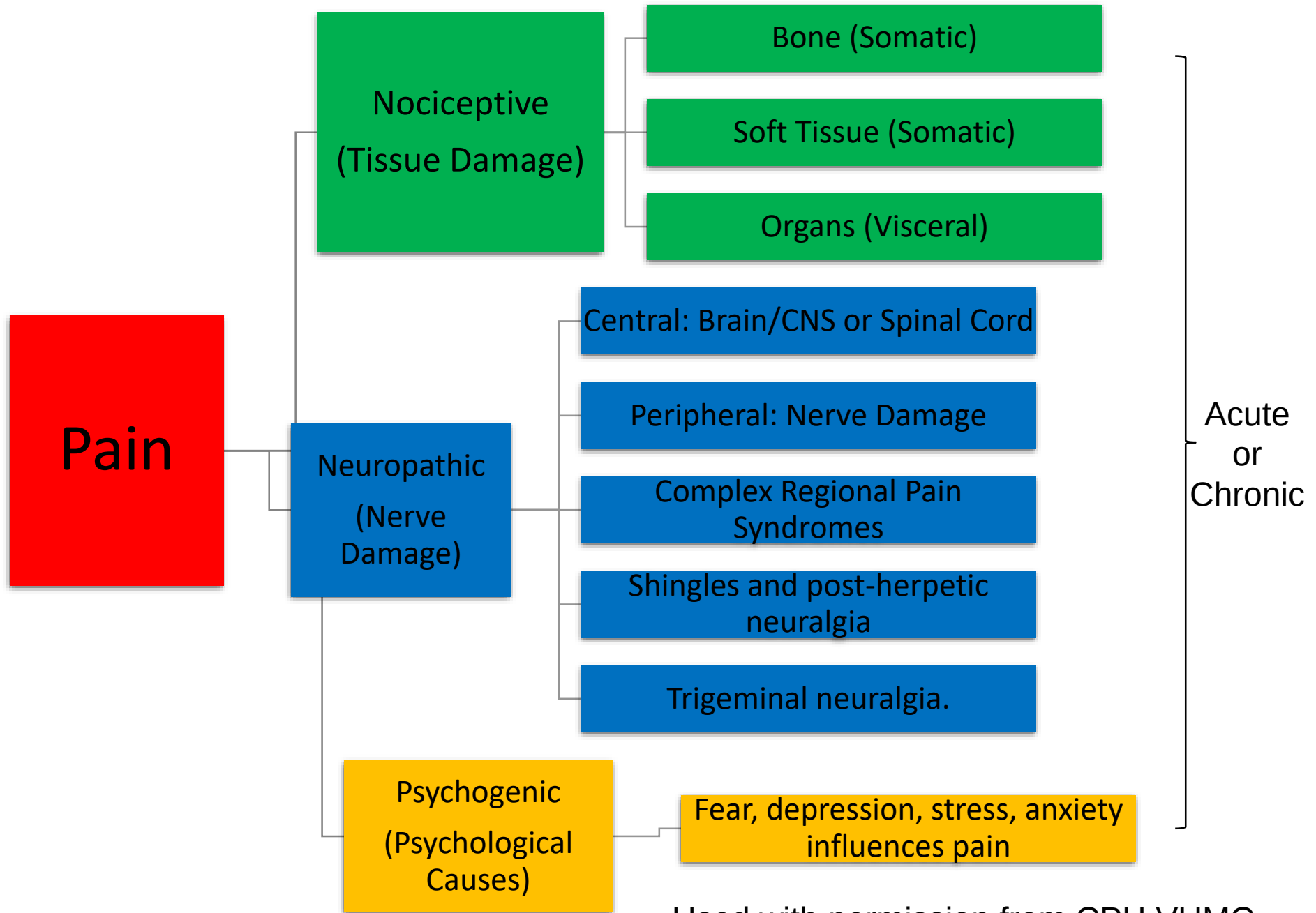
Pain Categories

Acute

The pain is expected to be over soon, but it can last seconds, weeks, or longer.

Chronic

The pain lasts beyond the healing of the causative injury and continues for several months. Commonly pain lasting > 3 months



Used with permission from CPH VUMC

Analgesic Medications

Acetaminophen

- 3 gm/day maximum

Side Effects.

- Hepatotoxicity.
- Ceiling effect in terms of analgesic efficacy.
- Little mood alteration

Analgesic Medications

NSAIDs / Salicylates:

- Aspirin.
- Propionic Acids: Ibuprofen, Naproxen.
- Indoles: Indomethacin.

Side Effects

- Ceiling effect in terms of analgesic efficacy.
- Little mood alteration.
- Erosion of protective mucus in GI tract.
- Nephrotoxicity

Antidepressant Medications for Pain Augmentation

Modulates the descending inhibitory pathways.
Improves sleep hygiene

- Tricyclic Antidepressants- TCAs.
 - Amitriptyline
- Serotonin-Norepinephrine Reuptake Inhibitors- SNRI's
 - Duloxetine

Anticonvulsant Medications for Pain Augmentation

- Voltage dependent Na^+ channel blocker.
 - Carbamazepine, oxcarbazepine, topiramate
- Voltage dependent Ca^{++} channel blockade.
 - Gabapentin, Pregabalin
- Voltage dependent Ca^{++} channel blockade & decreases the release of the glutamate, noradrenalin and substance P
 - Pregabalin

Topical Medications for Pain

Capsaicin.

- Derivative of Red Pepper
- Depletes substance P at nociceptive transmission

Local Anesthetics

- Lidocaine (topical, local or IV infusion)

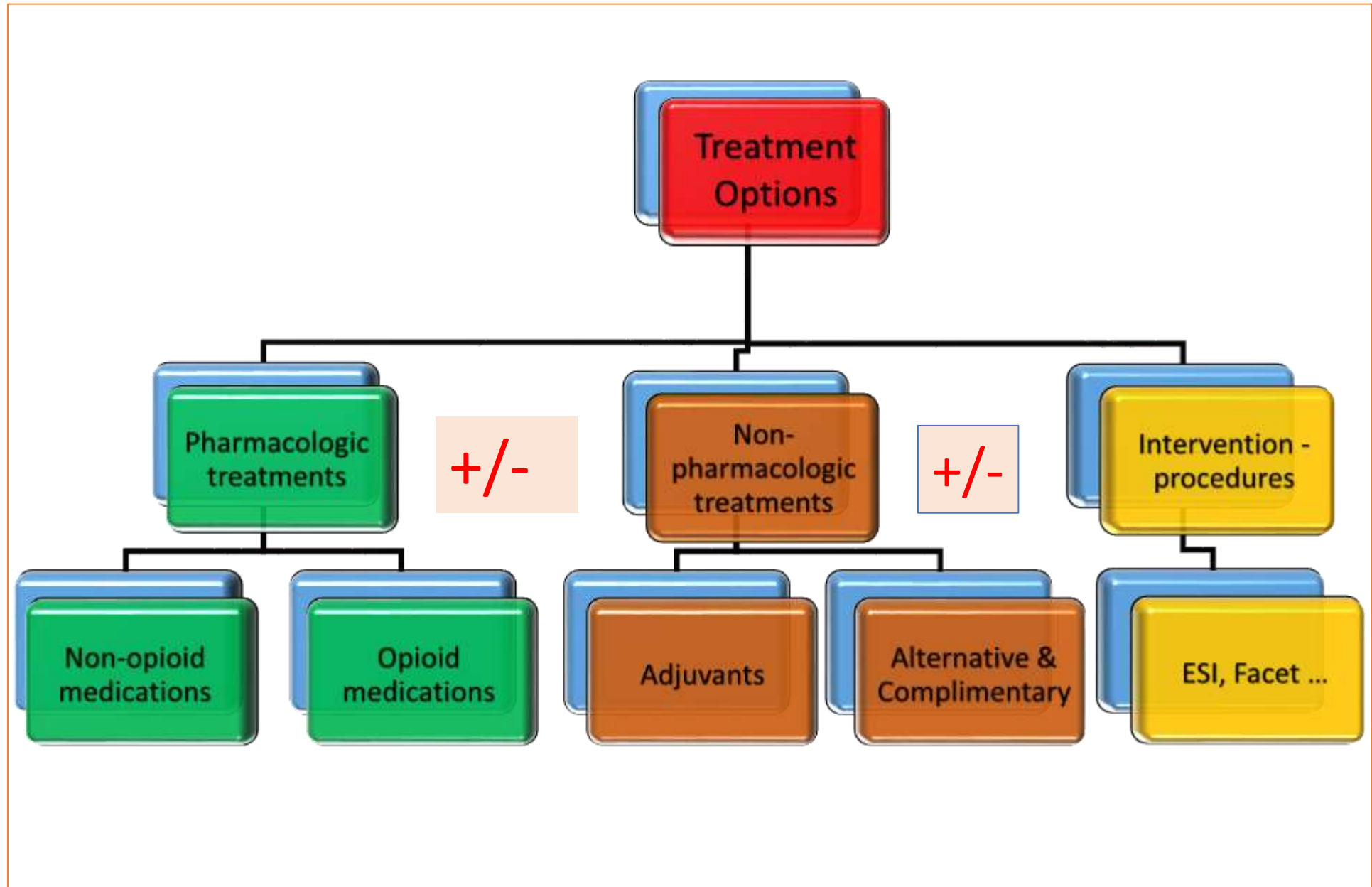
Non-Pharmacologic Interventions for Pain Augmentation

- Ice/heat
- Yoga/Tai Chi
- Chiropractic
- Biofeedback
- Acupuncture
- Hypnotherapy
- Mindfulness Based Stress Reduction
- Exercise/ Physical Therapy
- Cognitive Behavioral Therapies
- Numerous others

Interventional Procedures for Treatment of Pain

- Trigger Point Injection
- Tendon, bursa or intra-articular Injections
- Peripheral nerve blocks
- Sympathetic nerve blocks
- Epidural, Facet, Spinal injections
- Spinal stimulator
- TENS unit

Pain Control



Mis-prescribing

Prescribing in quantities or frequency inappropriate for the complaint or illness.

Examples:

- Large quantities/frequent intervals/crescendo pattern.
- Progression to multiple drugs.
- For trivial complaints.
- Family members.
- Known alcoholic or drug addict

Categories of Misprescribing

Dated: Fails to keep current.

Duped: Doesn't detect deception.

Dysfunctional: Can't say no.

Dismayed: Prescription is to make up for lack of time.

Dishonest: Prescribing for financial gain.

Disabled: Impaired judgment.

Disempowered: Skewed perception of power.

Disorganized: No systems in place.

Disregard for Scope: Practicing out of specialty.

Dodging: Using refills to avoid patient visits

Suspect Drug-Seeking Behavior in the Patient
who . . .



Possible Drug-Seeking Behavior

- A dramatic, compelling but vague complaint.
- Pressures for an increases in dose.
- Drug screens are negative for Rx'ed medicine.
- Symptoms contradict clinical observation.
- Patient asks for a specific drug.
- Patient has no interest in the diagnosis.
- Rejects all treatment that is not opioids

Possible Drug-Seeking Behavior

- Reports an NSAID Allergy.
- Has abundant pharmacologic knowledge.
- Patient makes veiled threats.
- Patient is very flattering.
- Primary doctor is out of town.
- Travels long distances to get to you.
- Primary doctor just retired

CLINICAL VIGNETTE

Dr. Cal Seeum sees her PCP for recurrent kidney stones. She rates her pain as a 7/10. Her PCP prescribes hydrocodone /APAP 10/325mg, #30 and refers her to the Urologist. The next morning the urologic workup shows 2 stones with no urinary obstruction. She was not asked about pain but was prescribed Hydrocodone/ APAP 10/325mg, #90. The patient was not asked by either office if she has taken any medication for pain or if she wants any pain medication or if she has pain medication at home. Dr. Seeum goes home with 2 Rx's of hydrocodone and takes APAP 1000mg and her pain score drops to 2.

CLINICAL VIGNETTE

- What are the red flags?
 1. The patient was prescribed hydrocodone twice without getting a prescription history, needing or wanting an opioid.
 - 2. Standing orders, quantities for 30 days, and other automatics are dangerous.**
 3. OTC medication worked for her pain

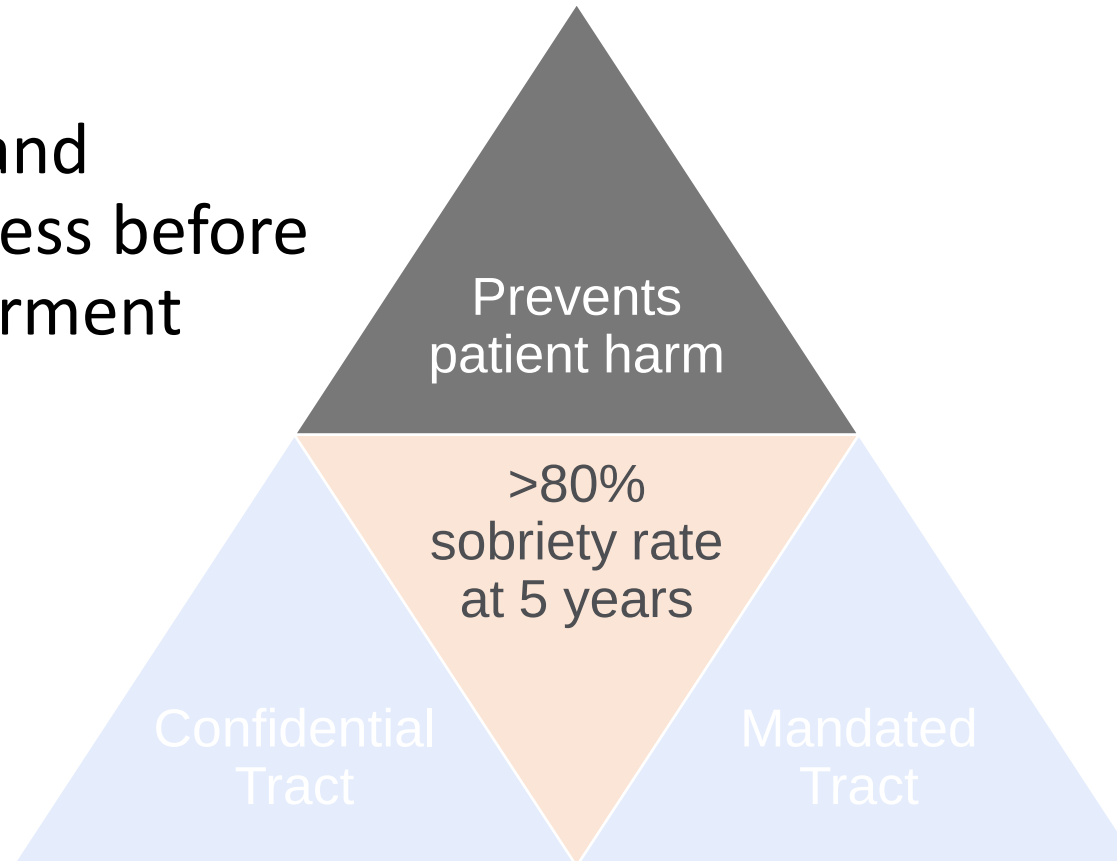
THERE IS HELP.

A **healthier** physician is a
better physician.



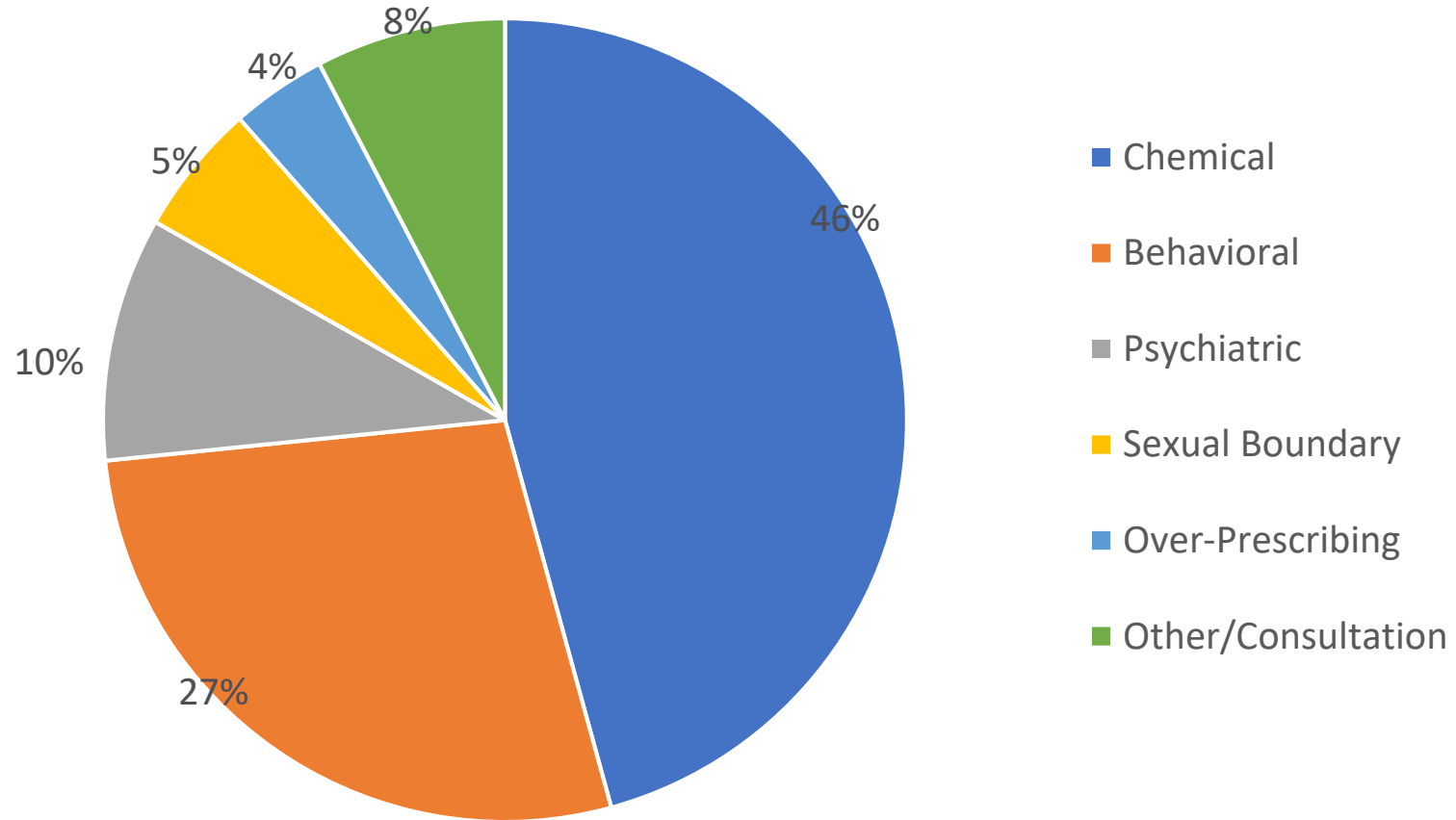
Tennessee Medical Foundation Physician's Health Program

Early detection and
treatment of illness before
functional impairment



TMF Identifications

January 2001 – December 2018



TN-PSQ

- An online mental health resource to address depression, burnout, and other mental health problems among Tennessee's licensed health professionals served by the Tennessee Medical Foundation's - Physician's Health Program (TMF-PHP).
- Open to all Tennessee physicians, residents, interns and medical students.
- Initiated by the TMF in partnership with the Board of Medical Examiners, the Tennessee Medical Association and State Volunteer Mutual Insurance Company

TN-PSQ

- TN PSQ is intended to help connect physicians with available mental health resources in their area.
- The TN PSQ is a free, anonymous, confidential online mental health screening to provide referrals to appropriate mental health resources and optional interaction with a program therapist.
- This tool is completely driven by the user. This should alleviate some of the fears involved in asking for help with mental or emotional illness.

TN-PSQ Website

tn.providerwellness.org

There are links on the TMF-PHP, TMA, BME and SVMIC websites.

Post Test: Questions

1. According to the International Narcotic Control Board the USA consumes what percentage of legitimately produced hydrocodone?
 - A. 1%
 - B. 24%
 - C. 49%
 - D. 74%
 - E. 99%

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Post Test: Questions

2. Opioid consumption increased in the USA from 1990 through 2010 because of?
 - A. More painful long bone fractures occurred.
 - B. Industry promotion of opioids to primary care doctors.
 - C. The institution of the 5th Vital sign.
 - D. A, B, & C
 - E. B and C only

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Post Test: Questions

3. The Overdose death rate in Tennessee has _____ each year from 2013 through 2018?
- A. Increased
 - B. Decreased
 - C. Not Changed
 - D. Increased but then decreased
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Post Test: Questions

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Post Test: Questions

4. The compulsive nature of Substance Use Disorders is caused by what organ?

- A. Spleen
- B. Liver
- C. Kidney
- D. Brain
- E. Heart

Post Test: Questions

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Post Test: Questions

5. The reward circuit includes the Ventral Tegmental Area and the _____?
- A. Triune Brain
 - B. Reptilian Brain
 - C. Nucleus Accumbens
 - D. NeoCortex
 - E. Nucleus Pulposus

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Post Test: Questions

6. Adverse Childhood Experiences (ACE) correlate with the development of ____?
- A. Obesity
 - B. Suicide Attempts
 - C. Anxiety
 - D. Substance Use Disorders
 - E. All of the above

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Post Test: Questions

7. A person that has developed an Alcohol Use Disorder in their early 20's had to have prior consumption of alcohol?
 - A. True
 - B. False

Post Test: Questions

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- A. True
 - B. False

Post Test: Questions

- 8. In Tennessee physicians are required to check the Controlled Substance Database (CSMD).
 - A. When prescribing a course of an antibiotic.
 - B. When prescribing a course of antibiotics for sepsis secondary to IV drug use.
 - C. When prescribing a new 5-day course of hydrocodone.
 - D. When prescribing a new 2-day course of oxycodone.

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Post Test: Questions

9. The Tennessee Chronic Pain Guidelines are meant only for Pain Management Specialists.
 - A. True
 - B. False

Post Test: Questions

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A. True

B. False

Post Test: Questions

10. Neonatal Abstinence Syndrome (NAS) is caused when _____ are consumed during pregnancy.

- A. Only illicit opioid drugs (like heroin)
- B. Only prescribed opioids (like oxycodone)
- C. Only prescribed or diverted buprenorphine and/or methadone
- D. All the above (A, B & C) can cause NAS.

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“Controlled Substance
Prescribing and the Tennessee
Chronic Pain Guidelines”

THANK YOU

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QUESTIONS?

“Controlled Substance Prescribing and the Tennessee Chronic Pain Guidelines”

W. Reeves Johnson, Jr., MD, FAAFP

Summit Medical Group

Knoxville, TN